



RESERVE
CLINIC

Peptide Market Audit, E-Commerce Strategy and Business Proposal

Proposed concept · Working draft for founders, partners, investors, clinicians and counsel
Prepared 2026-09-02 · United States market

Proposed principals: Scott Disick (brand partner, proposed) and Dr. Michael Azziz (medical cofounder, proposed). Both roles are unverified and subject to definitive agreements. No endorsement, ownership, or authorization is implied.

Contents

1 Confidentiality and important assumptions	4
Labeling convention	4
Research limitations that materially affect this document	4
Assumptions used throughout	5
2 Executive summary	5
What we recommend	5
Direct answers to the fifteen questions posed	7
3 The Reserve Clinic opportunity	8
Why now, and why this concept	8
What Reserve Clinic would sell	8
4 Market definition and market size	9
Which markets apply, and which do not	10
TAM, SAM and initial SOM (three definitions)	10
5 Market segmentation	11
6 Competitive landscape	12
Telehealth and clinician-led (top 12 of 27)	12
Other segments — leaders and cautionary cases	13
7 Top-performing website findings	13
Patterns reported across leaders	14
Customer-journey and medical-experience gaps noted	14
8 UI/UX patterns and white space	15
9 Product and pricing landscape	15
Regulatory classification of what is being sold	16
Monetization patterns and lifetime-value logic	16
10 Marketing and claims landscape	17
Narratives that win, clichés that saturate, claims that endanger	17
Acquisition channels observed	17
11 Celebrity-founder findings	18
Verified peptide-direct relationships	18
Lessons that shape the Reserve Clinic design	19
12 Reserve Clinic positioning	19
Why Territory A	20
Brand architecture	20

13 Scott Disick — proposed role	20
What the research found	20
Proposed responsibilities and terms (subject to definitive agreements)	21
14 Dr. Michael Azziz — proposed role	21
Credential requirements before appointment	21
Proposed responsibilities and protections	22
15 Recommended business model	23
Revenue architecture (legally supportable streams only)	23
16 Customer experience	24
17 Website concept	24
Design-system direction	24
Draft homepage copy (requires legal and medical review)	25
18 Sitemap and user flows	25
Primary flows	25
19 Sample wireframes	26
20 Technology and operations	27
Recommended stack and build/license/integrate split	27
21 Clinical and pharmacy partner model	28
Partner due-diligence framework	28
22 Compliance framework	29
High-risk issues and pre-launch actions	30
Medium and low issues (summary)	32
23 Launch strategy	32
Roadmap	33
24 Channel plan	33
Measurement framework (base-case month-12 targets [FA])	34
25 Financial model	34
Revenue and cost architecture	35
26 Scenario and sensitivity analysis	35
What matters most	36
27 Funding and resource requirements	37
Team, partners and approvals	37
Hypothetical brand-partner structures (no terms offered)	37
28 Milestones	38
29 Principal risks and mitigations	39

30 Go/no-go requirements	40
31 Recommended next steps	41
Decisions requiring founder approval	41
32 Sources and appendices	42
Appendix A — Selected sources	42
Appendix B — Deliverables	43
Appendix C — Verification backlog (priority 1)	43

1 Confidentiality and important assumptions

This document is a working draft prepared for the Reserve Clinic project owner. It combines a market audit, a strategic plan and a driver-based financial model for a **proposed concept**. It is not an offer of securities, and it is not legal, medical, regulatory, tax or investment advice. Every regulatory statement must be reviewed by qualified healthcare, FDA, pharmacy, corporate-practice-of-medicine, privacy, advertising and intellectual-property counsel before reliance.

Labeling convention

Label	Meaning
Verified fact	Corroborated against an authoritative or primary source surfaced in research on 2026-09-02
Company-reported claim	Stated by the company itself (site, release, filing); not independently verified
Third-party estimate / report	Trade press, law-firm alert, database or estimation tool; not independently verified
Analyst inference	Our synthesis from indirect evidence
Strategic recommendation [SR]	What we advise; a judgment, not a fact
Financial assumption [FA]	A modeling input; editable in the workbook; not a forecast
Recalled / unverified	Figures the research team could not access live and reproduced from prior knowledge; must be re-checked

Research limitations that materially affect this document

The research environment used for this project blocked direct page loads of company websites, regulators, review sites, advertising libraries and traffic tools, and capped web searches. Consequently: (1) company facts derive from dated search-result excerpts of the cited pages rather than full page reads; (2) website UI/UX scores are either 'not assessed' or low-confidence inferences, and no screenshots were captured; (3) traffic estimates, social audience counts and ad-library activity were not retrievable; (4) the market-size and benchmark workstream was produced without live access and is labeled recalled/unverified; (5) identity, licensure and trademark checks for the proposed principals could not be completed against USPTO, NPI Registry or state medical boards. Nothing was invented. Every deliverable carries a verification backlog (see 08_ReserveClinic_Evidence_Ledger.xlsx) that should be worked before any external use.

Assumptions used throughout

- Initial geography: United States; prescription programs launch only in states where corporate-practice, telehealth-modality and pharmacy-licensure conditions are satisfied.
- Primary model evaluated: premium consumer health — education, clinical intake, telehealth through an affiliated medical practice, licensed-pharmacy fulfillment, membership, and compliant non-prescription commerce.
- Brand and domain: Reserve Clinic / ReserveClinic.com, adopted after the research phase in place of the AminoLord working name (the research files therefore use the earlier name; principal and market findings are unaffected). reserveclinic.com resolves to registrar parking addresses, consistent with a registered but unlaunched domain (analyst inference from DNS). No public entity, trademark filing, social presence or press mention was found for either name; registrar and USPTO checks were blocked. Note for counsel: some states restrict use of 'clinic' in the name of an entity not owned by licensed professionals, so which entity may trade under the name must be confirmed.
- Scott Disick and Dr. Michael Azziz: proposed roles only. No public evidence connects either to Reserve Clinic. Two scenarios are planned: A (named partners under signed agreements) and B (independent brand).
- Budget, ownership, launch date and capitalization are unconfirmed; the financial model provides conservative, base and upside cases with an assumptions register.

2 Executive summary

Peptides moved from niche to mainstream between 2023 and 2026, pulled by GLP-1 normalization, podcast culture and a 2026 federal reversal that is reopening compounding access to several popular peptides. The category is now crowded with three very different kinds of business that are often confused: licensed telehealth programs, research-use-only (RUO) chemical vendors, and consumer 'peptide' supplements and cosmetics. Enforcement in 2025–2026 has been aimed squarely at the second group and at misleading compounded-GLP-1 marketing in the first. The white space is a brand that is premium in feel, clinician-led in substance, and radically transparent about what it will not sell.

109

companies mapped;
75 audited in depth

59

celebrity relationships
classified

36

compliance issues; 23
rated High

\$42.8M

Base-case year-3 net
revenue [FA]

\$4.8M

Base-case capital
required [FA]

What we recommend

- **Business model:** a premium membership and clinician-led telehealth platform (management-services company plus affiliated medical practice and licensed 503A/503B pharmacies), with an education library and a separate non-prescription shop. Prescription products are reached only through eligibility screening and independent clinician consultation, never an open cart. Compounded GLP-1s are excluded; GLP-1 demand is routed to branded direct-to-consumer channels or clinician care.
- **Positioning:** 'Peptide health, done properly' — standards-first premium longevity for 30–55-year-olds who have heard about peptides and want a trustworthy way in. Reasons to

believe: named licensed clinicians, licensed US pharmacies, an FDA-status badge on every ingredient, honest evidence grades, all-in pricing, and a public list of what Reserve Clinic will not sell.

- **Principals:** proceed on two parallel tracks. Scenario A treats Scott Disick as a brand partner for creative direction and launch awareness under a name-and-likeness agreement with FTC disclosure, morality, exclusivity and termination terms, and Dr. Michael Azziz as medical director once identity, licensure and board status are verified. Scenario B launches without either. The brand must be able to stand on its clinical substance in either case.
- **Launch path:** six phases over roughly nine months to public launch: foundation and compliance, internal pilot, private beta in 4–6 states, waitlist and early access, coordinated public launch, then state-by-state expansion. The base case reaches positive monthly EBITDA in month 18 and needs about \$4.8M of capital; the conservative case needs \$12.9M and does not break even within 36 months, which is the honest downside if demand disappoints and spend continues.
- **Largest risks:** regulatory reversal on peptide compounding; FTC action on subscriptions, testimonials and endorsements; state corporate-practice and pharmacy-licensure constraints; ad-platform and payment-processor restrictions; and key-person and reputation risk concentrated in a celebrity partner with a prior FTC warning letter and a documented GLP-1 association.

Direct answers to the fifteen questions posed

Question	Short answer
Segment leaders	Telehealth: Hims & Hers (scale), Function Health (capital, no Rx yet), Superpower, Hone, Joi/Blokes. Pharmacies: Belmar, Wells, Olympia (clean records; dual 503A/503B). RUO: none recommended; category is under enforcement. Consumer: Thorne, Vital Proteins, Medik8, The Ordinary. Platforms: Function, Superpower, SSRP (education).
Strongest UI/UX and conversion systems	Could not be scored from rendered pages this session. Third-party descriptions point to flat transparent pricing (Peter MD, Henry Meds), labs-first onboarding (Hone, Marek, Superpower), and Hims-style bundled convenience as the conversion patterns to study first.
Most attractive categories and models	Clinician-led programs built on lawfully compoundable or approved peptides (sermorelin, NAD+, tesamorelin) plus diagnostics memberships; RUO and compounded GLP-1 are commercially attractive only until enforcement, which has already arrived.
Effective marketing tactics	Founder or clinician long-form content, podcasts, educational SEO, labs-to-program funnels, disclosed creator programs; single-spectacle campaigns produce spikes not trust.
Riskiest practices	RUO positioning, compounded-GLP-1 'same as Ozempic' claims, hidden subscriptions and fees, fake or atypical testimonials, before/after imagery, undisclosed celebrity compensation, health-data leakage to ad pixels.
Verified celebrity founders/partners	29 verified founders/cofounders and 14 verified investors across 59 records (e.g., Kourtney Kardashian/Lemme, Hailey Bieber/Rhode, Mark Hyman/Function, Robbins & Diamandis/Fountain Life, Campbell & Williams/BioLongevity Labs; Serena Williams is a Ro ambassador, not an owner).
Is celebrity-led positioning durable?	Only when the celebrity owns equity, engages repeatedly in trusted formats, and the product has independent substance. Cosmetic involvement (Ladder, SKKN) did not convert; exits (Oprah/WW, Brecka/10X) were costly.
Copy / adapt / avoid	Copy: transparent all-in pricing, named clinicians, labs-first safety. Adapt: diagnostics membership, education library. Avoid: RUO, ingredient 'stacks', step-up intro pricing, hidden memberships, B/A imagery.
Business model to pursue	Option 3 hybrid: premium membership plus telehealth/MSO with compliant commerce; education-only as fallback if counsel restricts the formulary.
What must be true before principals are public	Verified identity and credentials; signed agreements with NIL, disclosure, morality, exclusivity, termination and post-termination terms; resolution of any prior GLP-1 endorsement exclusivity; medical-independence protections; counsel sign-off.
Website architecture and technology	Next.js site and member portal; Shopify for non-Rx commerce only; licensed telehealth/EHR/e-Rx vendor for the clinical core; Stripe Billing for programs and membership; HIPAA-eligible data warehouse; consent logging; no health data to ad platforms.
Most credible launch sequence	Foundation → internal pilot → private beta (4–6 states) → waitlist and early access → coordinated public launch → tranche-based expansion.
Capital, team, partners, approvals	Base \$4.8M (conservative \$12.9M); 10–14 people by launch; telehealth/EHR vendor, 2 pharmacies, lab partner, ID verification, high-risk-capable processor; counsel opinions, LegitScript certification, insurance, medical director sign-off.
Biggest reasons for failure	Regulatory reversal; FTC/state enforcement; ad-platform lockout; retention below 85% month-1; principal controversy; pharmacy quality event; running the conservative case without cutting spend.
Five next actions	1) Verify principals and secure term sheets or activate Scenario B; 2) engage counsel for structure and formulary memo; 3) file trademark clearance and secure handles; 4) issue vendor RFPs (telehealth/EHR, pharmacies, labs, payments); 5) fund Phase 0 and start the prototype and library.

3 The Reserve Clinic opportunity

Three forces converge. First, demand: peptides are now a mainstream wellness conversation, carried by GLP-1 normalization (roughly one in eight US adults reports ever using a GLP-1 drug per KFF polling — recalled, unverified), by podcast hosts who have discussed BPC-157 dozens of times, and by a 2025–2026 press cycle on the 'peptide boom'. Second, supply: the Sept 2023 FDA decision that placed about nineteen popular peptides in 503A Category 2 was partially reversed in 2026 — HHS announced on Feb 27, 2026 that most would move out; FDA updated the 503A bulks document on Apr 15, 2026; and the Pharmacy Compounding Advisory Committee recommended BPC-157, KPV, TB-500, MOTS-c, semax and epitalon on Jul 23–24, 2026 over staff objections (third-party reports; final rulemaking pending). Third, trust: enforcement against RUO sellers and misleading compounded-GLP-1 marketing has removed the low-integrity end of the market and made compliance a visible differentiator.

Why now, and why this concept

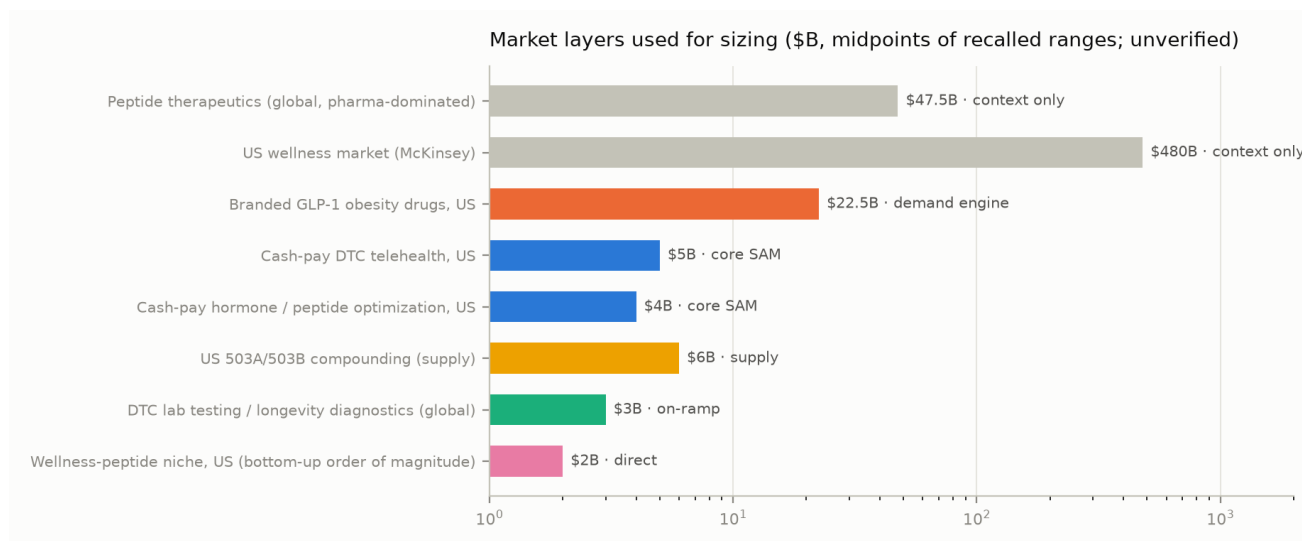
- The incumbents are either scaled generalists adding peptides late (Hims & Hers plans NAD+, sermorelin and glutathione before end-2026), diagnostics-first platforms converging on the same funnel (Function, Superpower, Hone, Lifeforce), or budget hormone clinics with thin trust infrastructure. Nobody owns 'the trustworthy premium front door' to peptide health.
- A brand that publishes FDA status and evidence grades for every ingredient, names its clinicians and pharmacies, prices all-in, and states what it will not sell can convert the largest untapped segment: curious, affluent adults who are put off by both the 'peptide bro' aesthetic and the hospital feel.
- Celebrity awareness can compress the time to a national audience, but the research is unambiguous that awareness without substance does not convert and that a health brand inherits its face's reputation. The concept is therefore designed so that the medical substance leads and any celebrity involvement amplifies it.

What Reserve Clinic would sell

Layer	Offer	Legal frame	Revenue
Education	Peptide library, evidence grades, clinician video, journal	Editorial; medically reviewed	Indirect (SEO, trust)
Membership	Navigation, clinician Q&A, lab coordination, member pricing on non-Rx, community	Membership services; auto-renewal law	Recurring fee
Clinician-led programs	Eligibility → intake → independent clinician → Rx if appropriate → licensed pharmacy → monitoring	Affiliated medical practice; MSO services; pharmacy dispensing	Program fees (structure per counsel)
Labs	Baseline and monitoring panels via lab partners	Clinician-ordered	Lab fees
Non-Rx shop	Supplements and cosmetics that meet standards; no RUO	DSHEA / cosmetics	Retail margin

4 Market definition and market size

Caveat on every number in this section: the market-size workstream could not access syndicated reports or filings during this project; values are recalled ranges from analyst knowledge (cutoff mid-2026), labeled recalled/unverified in the evidence ledger with source URLs to re-check. They are presented for structure and order of magnitude, not as verified data.



Source / label: research/06_market_size.json (recalled, unverified): Grand View Research, Precedence Research, Fortune Business Insights, McKinsey Future of Wellness 2024–25, company statements. Midpoints of ranges; log scale.

Which markets apply, and which do not

Market	Definition, geography, base, forecast, CAGR	Reported or inferred	Applicability
Peptide therapeutics	Approved peptide drugs; global; 2024 ~\$45-50B → 2030 ~\$75-80B; ~8-9%	Third-party (recalled)	Low — pharma GLP-1/insulin dominated; context only
503A/503B compounding	US; 2024 ~\$5.5-6.5B → 2030 ~\$8-9B; ~5-6%	Third-party (recalled)	High as supply market and cost base
Telehealth / virtual care	US; 2024 ~\$40-45B → 2030 ~\$90-120B; 15-20%; cash-pay DTC subset ~\$4-6B (2025)	Third-party; DTC subset inferred	High — cash-pay DTC subset is the core SAM
Longevity / healthy aging	Global anti-aging ~\$70-80B (2024); longevity therapies ~\$25-30B; ~7-9%	Third-party (recalled)	Moderate — demand narrative
Medical weight management / GLP-1	US branded anti-obesity ~\$20-25B (2025) → \$60-80B (2030); compounded GLP-1 peak run-rate ~\$1.5-3B early 2025, declining	Inferred from Novo/Lilly, Hims, LifeMD	High as demand engine; compounded segment is a wind-down, not a target
Hormone optimization / TRT	US TRT ~\$1.2-1.5B (2024); cash-pay hormone/peptide clinics and telehealth ~\$3-5B (2025), 12-15%	Third-party; clinic set inferred	High — direct competitive set
Sports recovery / performance	US recovery tech and services ~\$3-5B; sports nutrition global ~\$45-50B	Inferred / third-party	Moderate — adjacency
Consumer wellness	Global ~\$2.0T (2025); US ~\$480-500B; 4-10%	Third-party (McKinsey, recalled)	Context only
Premium / personalized supplements	US supplements ~\$55-65B; personalized nutrition global ~\$15-20B, 12-15%	Third-party (recalled)	Moderate — non-Rx shop
Personalized / precision medicine	Global ~\$550-650B broad; ~\$100-140B narrow	Third-party (recalled)	Low — excluded from sizing
Men's health telehealth	US ~\$3-4B (2024) → \$6-8B (2028)	Inferred from Hims, Ro, Hone	High — same persona
DTC lab testing / diagnostics memberships	Global ~\$2.5-3.5B (2024) → \$5-7B; Function Health ~\$2.5B valuation, >200k members at \$499/yr	Third-party; company-reported	High — on-ramp product

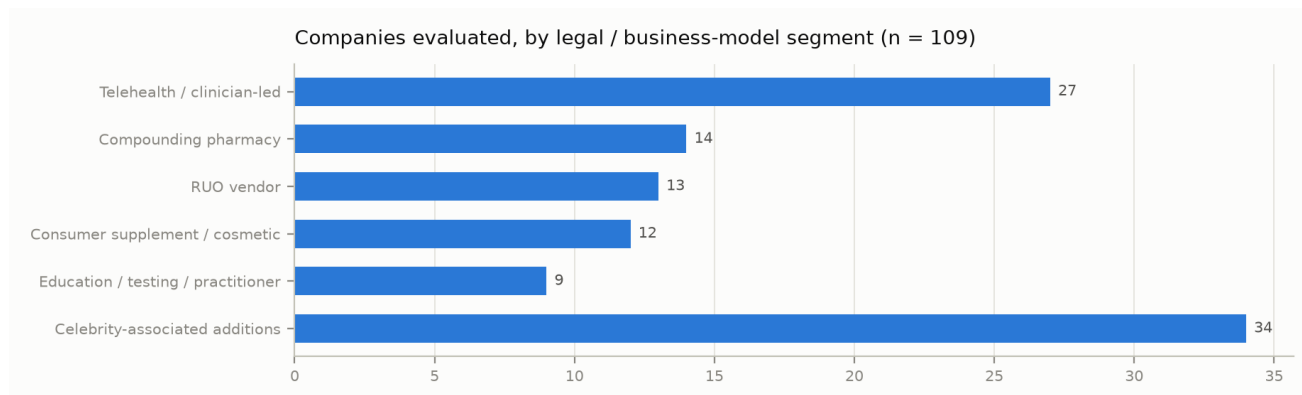
TAM, SAM and initial SOM (three definitions)

	Conservative	Base	Expansive
TAM (US, annual spend addressable by the model)	Cash-pay hormone/peptide optimization (~\$3B) + cash-pay DTC telehealth share relevant to longevity/men's health (~\$2B) + longevity diagnostics memberships (~\$1B) ≈ \$6B	Add full cash-pay DTC telehealth (~\$5B) and premium supplement slice (~\$3B) ≈ \$11B	Add branded GLP-1 DTC context and broader premium wellness ≈ \$20B+
SAM (states served × premium segment)	40% population coverage at launch × ~\$4B direct set ≈ \$1.6B	85% coverage by month 24 × ~\$5B ≈ \$4B	National coverage × ~\$8B ≈ \$8B
SOM (year-3 net revenue from the model)	\$5.1M (~0.3% of SAM)	\$42.8M (~1% of SAM)	\$194.2M (~2.5% of SAM)

Source / label: Analyst inference built on recalled ranges above and the financial model (07_ReserveClinic_Financial_Model.xlsx). All three definitions must be rebuilt once syndicated reports and company filings are verified.

5 Market segmentation

The seven categories below operate under different laws and economics and should never be compared as if equivalent. The universe mapped in 01_Peptide_Market_Landscape.csv contains 109 companies; 75 received the detailed audit.



Source / label: 01_Peptide_Market_Landscape.csv; research/01, 02, 03. Counts are companies profiled, not market shares.

Segment	How revenue is made	Rx / clinician?	Who dispenses	Product status	Journey and pricing	Principal risks
1 Licensed telehealth / clinician-led	Program or medication subscriptions, consults, memberships	Yes	Partner or owned 503A/503B pharmacies	Approved and compounded drugs	Quiz → intake → async/video → Rx → ship; \$79-\$400/mo + \$39-\$199 memberships	CPOM, FTC subscriptions/claims, compounded-GLP-1 enforcement, formulary changes
2 Compounding pharmacies	Dispensing fees on prescriptions; 503B bulk sales to clinics	Yes (prescriber)	Themselves	Compounded preparations	B2B; not consumer facing	FDA 483s/warning letters, sterility, bulks-list status, brand-owner litigation
3 Longevity, hormone, metabolic, performance clinics	Memberships \$10K-\$85K/yr (Fountain Life) to \$99-\$350/mo clinics; labs; Rx	Yes	Partner pharmacies	Approved and compounded	Labs-first; concierge	Claims, off-label promotion, key-person
4 Research-use-only vendors	Online sale of peptide vials 'for research'	No	Themselves (often foreign manufacturing)	Unapproved drugs when marketed for human use	Cart checkout; alternative payment rails; affiliate codes	FDA warning letters (Mar 2026 wave), DOJ prosecutions (Amino Asylum, Paradigm), card-network bans
5 Consumer supplement, collagen, cosmetic peptide	Retail and subscription product sales	No	3PL / retail	Supplements and cosmetics	Standard e-commerce; \$16-\$70 AOV	DSHEA/cosmetic claims; 'GLP-1' naming (Lemme class actions)
6 Education, membership, testing, practitioner platforms	Diagnostics memberships \$149-\$499/yr; certifications \$99-\$1,000; communities	Sometimes (labs ordered by clinicians)	Labs / n/a	Services	Annual membership; upsell to care	Health-data privacy; unlicensed protocol advice in communities

Segment	How revenue is made	Rx / clinician?	Who dispenses	Product status	Journey and pricing	Principal risks
7 Celebrity-founded adjacent wellness	Product sales leveraging founder audience	Rarely	3PL / retail	Supplements, cosmetics, fitness	DTC and retail	Endorsement disclosure, substantiation, key-person

6 Competitive landscape

Ranking method: because private revenue and traffic tools were unavailable, companies were ranked within their own segment on a transparent proxy scorecard — disclosed funding or performance, formulary and program breadth, pricing evidence, review signals and vertical integration — with traffic, advertising and social metrics recorded as not retrieved. Ranks are provisional and documented in 02_Competitor_Website_Audit.xlsx (Segment Rankings).

Telehealth and clinician-led (top 12 of 27)

#	Company	Why it ranks here (evidence label)	Peptide relevance
1	Hims & Hers	Only public scaled player (\$753M Q2-2026 revenue, company-reported via press); owns pharmacies; peptides (NAD+, sermorelin, glutathione) announced for launch before end-2026 with lab testing	Entering; waits for FDA clearance on restricted peptides
2	Function Health	\$2.5B valuation (Nov 2025) and \$450M growth financing (Jul 2026); >100k-200k members at \$499/yr; celebrity investors; acquisitions (Ezra, Getlabs)	No prescribing yet; obvious entrant
3	Superpower	\$199-\$499/yr diagnostics; 13 Rx live since Jan 2026 incl. sermorelin and NAD+; ~\$72M raised; celebrity investors	Direct
4	Hone Health	~\$63M 2025 revenue (third-party estimate); \$33M Series A; lab-gated sermorelin \$130/mo + \$149 membership; strong content	Direct
5	Joi + Blokes	>\$50M bootstrapped revenue, 50k+ patients; sermorelin ~\$199/mo	Direct
6	Mochi Health	500k+ patients, cash-flow positive (company-reported); sermorelin and NAD+	Direct
7	Ivím Health	Inc. 5000 #38 (2026); 50 states; sermorelin ODT \$129, NAD+ \$249, membership \$74.99	Direct
8	Noom Med	Acquired Tailor Made Compounding (Apr 2026; 40+ peptide catalog); retail rollout not yet verified	Vertical integration
9	Peter MD	Low-price leader (\$79-\$99 TRT; peptides \$150-\$400); ~14k Trustpilot reviews at 4.8; sells BPC-157 in transitional status	Direct; compliance watch
10	Maximus	Brand-led; \$15M Series A; sermorelin <\$175/mo	Direct
11	Lifeforce	\$129-\$199/mo longevity membership with quarterly labs and peptides; closest single analog	Direct
12	Marek Health	Influencer-founded (MPMD); premium labs-first; broad peptide menu incl. BPC-157; per-vial pricing (~\$350/15 mg)	Direct; compliance watch

Source / label: research/01_telehealth_companies.md and .json; labels per row in 08_ReserveClinic_Evidence_Ledger.xlsx. Ro (GLP-1 only), Eden, Henry Meds, TRT Nation, Defy, Gameday, AlphaMD, Calibrate, Limitless Male, BodyLogicMD, Fountain, Concierge MD, Mito, Found and Craft complete the 27.

Other segments — leaders and cautionary cases

Segment	Leaders (rationale)	Cautionary cases
Compounding pharmacies (14)	Belmar (6x503A + 2x503B, hormone-anchored, no enforcement found); Wells (503A+503B, in-house sterility testing); Olympia (dual model, compliance-forward content); Red Rock (PCAB, LegitScript)	Empower (warning letters 2017, 2021, two in Apr 2025; Lilly litigation); ReviveRx (WL Sept 2025 on HCG/TB-4); Boothwyn (WLs 2018, Jan 2026; recall); ProRx (WL Mar 2025; recall); Tailor Made (2020 WL, guilty plea, \$1.79M forfeiture; now Noom-owned)
RUO vendors (13; 3 defunct)	None recommended. Particle Peptides and BioLongevity Labs publish the strongest quality claims; both still market to consumers, which FDA treats as intended human use	Peptide Sciences (voluntary shutdown); Amino Asylum (raid Jun 2025; guilty pleas Dec 2025); Paradigm Peptides (owner sentenced ~6 years, Aug 2026); Swiss Chems (WL Dec 2024); PureRawz (WL Sept 2025)
Consumer supplement / cosmetic (12)	Thorne (\$3.8B P&G acquisition pending, Aug 2026); Vital Proteins (#1 collagen, Circana); Medik8 (L'Oréal ~€1B; peptide skincare); The Ordinary (copper peptides ~\$31); Momentous (NSF, Huberman)	Oral 'BPC-157' capsule brands (not a lawful dietary ingredient; availability contracting); Lemme GLP-1 Daily (class actions; rebranded)
Education / testing / practitioner (9)	Function Health; Superpower; SSRP Institute (transparent clinician curriculum); InsideTracker; SiPhox	Consumer communities selling protocol advice without licensure (affiliate-driven demand for RUO)

Source / label: research/02_pharmacy_ruo_consumer.md; FDA warning-letter references as cited in the evidence ledger (Verified fact where the FDA page surfaced; otherwise Third-party report).

7 Top-performing website findings

Honest status: no competitor website could be rendered during this project, so the 23-criterion scores in 02_Competitor_Website_Audit.xlsx are marked 'not assessed' or low-confidence inference, and no screenshots exist. The findings below are patterns reported consistently by third-party reviews, press and search excerpts. They should be confirmed in a rendered-page audit (checklist provided in the workbook) before being treated as fact.

Patterns reported across leaders

Pattern	Where reported	Effect on conversion or trust	Reserve Clinic stance [SR]
Flat, published pricing	Peter MD (\$79-\$99), TRT Nation (\$99), Henry Meds (\$149, no membership), Hone (\$130 + \$149)	Reduces friction; but membership add-ons (lvim ~\$75, Eden \$39→\$99, Hone \$149) often double effective cost and draw complaints	All-in pricing before intake; one optional membership, never required for Rx pricing
Labs-first onboarding	Hone (owned phlebotomy), Marek, Superpower, Function, LifeForce	Improves safety narrative and retention; adds cost and delay	Labs where clinically indicated; transparent lab pricing
Bundled convenience	Hims (consult + Rx + ongoing testing in one price)	Highest scale in category	Bundle consult, plan, monitoring; medication price shown separately when required by counsel
Curated review widgets	TRT Nation: Trustpilot 1.9 vs Birdeye 4.9	Divergence is a due-diligence flag and an FTC fake-review exposure	Verified reviews only; publish aggregate and link to source
Step-up introductory pricing	Eden, Noom	Boosts starts; ROSCA disclosure hotspot	Avoid
Peptide 'menus' with per-vial prices	Defy (34 peptides from \$160/mo), Marek (BPC-157 ~\$350/vial)	Signals breadth; risks reading as a catalogue of unapproved drugs	Programs by health goal; ingredient library with FDA status instead of a menu
Compliance-forward waiting	AlphaMD and Hims explicitly wait for FDA clearance before adding restricted peptides	Builds trust with clinicians and regulators	Adopt and publicize
Founder-influencer channels	Marek (MPMD), Ways2Well (Rogan appearances), BioLongevity Labs	Powerful demand; scrutiny attaches to the founder	Clinician-led content first; celebrity amplifies

Customer-journey and medical-experience gaps noted

- Cancellation terms and refill criteria were rarely described in accessible third-party sources; FTC's NextMed order (Dec 2025) and the 2026 FTC/state action against Hims & Hers (third-party report) show these are enforcement targets.
- Clinician credentials are frequently presented generically ('our doctors'); named-clinician transparency (Defy, Hone) is the exception.
- Adverse-event instructions, contraindication screening and emergency disclaimers are not prominent on landing pages described in reviews; they are typically inside FAQs or consent documents.
- Several clinics sold BPC-157 during its transitional status (Apr-Sep 2026) — a visible signal of how each company weighs regulatory risk.

Source / label: research/01_telehealth_companies.json (customer_journey_notes, medical_experience_notes); research/05_regulatory_brief.md. Analyst inference where noted.

8 UI/UX patterns and white space

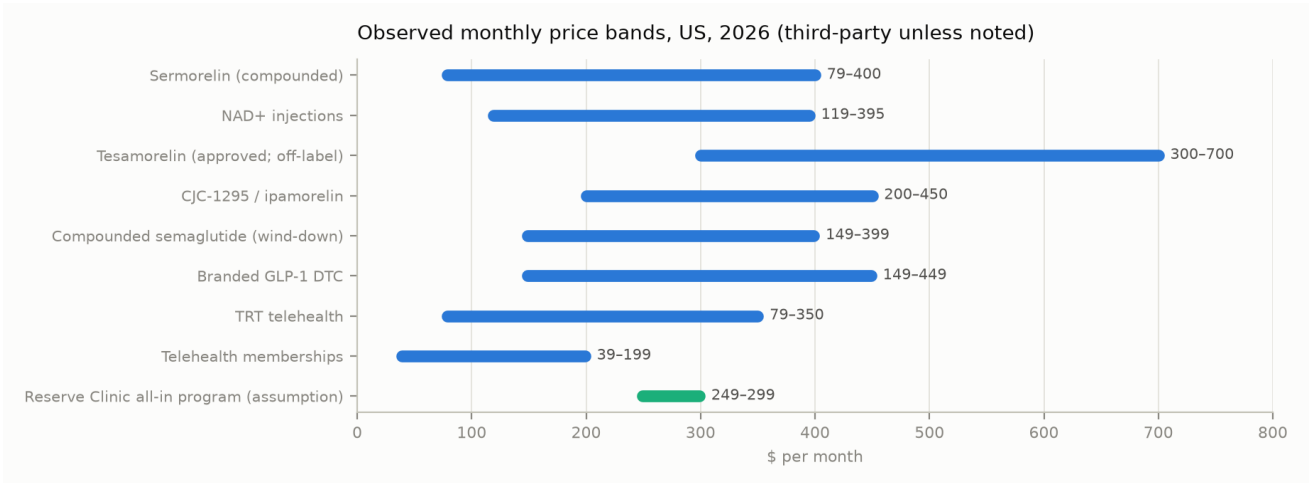
The category splits into two visual worlds: budget hormone clinics and RUO vendors with syringe imagery, dark gradients and price-led layouts; and diagnostics platforms with clean, data-heavy dashboards. Neither speaks to the premium, cautious adult who wants to understand before committing. The white space is an editorial-health experience that makes standards visible.

White-space opportunity	Evidence of gap	How Reserve Clinic fills it
FDA-status and evidence badge on every ingredient	No audited site is reported to label ingredients by 503A category or evidence grade	Library component with 'Approved / Compounded / Restricted — not offered / Supplement' badge and A-D evidence grade with citations
'What we won't sell' transparency	Compliance postures are implicit (AlphaMD, Hims) rather than marketed	Homepage block and Standards page listing exclusions (RUO, Category 2, compounded GLP-1s)
Separation of Rx and retail	Mixed menus at clinics; RUO carts	Programs use 'Check eligibility'; only non-Rx items have 'Add to bag'
Named clinicians with licensure	Generic 'our doctors' common	Clinician cards with license states and independence statement
Two-click cancellation and renewal reminders	Widespread opacity; ROSCA enforcement	Subscription page designed to exceed California ARL and FTC expectations
Health-data-aware analytics	FTC actions on health-data sharing (GoodRx, BetterHelp precedents; Hims 2026 third-party report)	Server-side pipeline; no health inputs to ad pixels; consumer-health-data consent
Premium, discreet tone	Biohacker jargon and physique imagery dominate	Editorial typography, real people, no before/after

Source / label: Analyst inference from research/01 and research/02 messaging and audit notes; 05_ReserveClinic_Website_Strategy.md sections 1-2.

9 Product and pricing landscape

136 offerings were inventoried across the 75 audited companies (03_Peptide_Product_and_Pricing_Audit.xlsx). Prices below are third-party or company-reported values captured on 2026-09-02; RUO vendor prices were deliberately not recorded because the sites could not be loaded.



Source / label: 03_Peptide_Product_and_Pricing_Audit.xlsx (Price Bands); research/01, research/06 (benchmarks recalled/unverified for tesamorelin and CJC/ipamorelin). Reserve Clinic band is a financial assumption.

Regulatory classification of what is being sold

Classification	Examples found in market	Implication for Reserve Clinic
FDA-approved drugs	Semaglutide, tirzepatide (branded DTC \$149-\$449); tesamorelin (Egrifta); bremelanotide (Vyleesi); testosterone	Prescribable; brand must not promote off-label uses
Compounded, permissible (Category 1 or approved-drug API)	Sermorelin, ipamorelin, NAD+, glutathione	Core of an initial formulary, subject to counsel and medical director
Transitional (removed from Category 2 Apr 2026; PCAC-recommended Jul 2026; final rule pending)	BPC-157, KPV, TB-500, MOTS-c, semax, epitalon; CJC-1295, GHK-Cu inj., AOD-9604, Ta1, selank pending review	Exclude until final rulemaking; publish library entries; monitor monthly
Remaining Category 2	Melanotan II, GHRP-2, GHRP-6, LL-37, PEG-MGF	Never
Compounded GLP-1s	Semaglutide \$149-\$399; tirzepatide \$299-\$599; 100+ FDA warning letters 2025-26; Apr 30, 2026 proposal to exclude from 503B bulks list	Exclude; route to branded DTC or clinician care
Supplements / cosmetics	Collagen peptides, copper-peptide serums, urolithin A	Non-Rx shop candidates with substantiated claims
RUO	Any vial 'for research' sold to consumers	Never sold, linked or described as available

Monetization patterns and lifetime-value logic

- Memberships (\$39-\$199/mo) and care plans are the dominant recurring layer; they are also the most criticized when hidden. Diagnostics platforms anchor on annual fees (\$149-\$499).
- Consultation fees range from \$0 (bundled) to \$250 (Defy); lab fees are bundled (Hims planned) or \$99-\$600 separately.
- Lifetime value is retention-driven: benchmarks (recalled) suggest 5-10% monthly churn for DTC health subscriptions, 8-15% for GLP-1 and 2-4% for hormone programs.
- Pricing recommendation [SR]: do not copy competitor prices. Run the ten-step pricing plan (Van Westendorp, MaxDiff, interviews, landing-page tests, cost floor with $\geq 55\%$ gross margin) in Phase 1; the model's \$249-\$299 all-in program band is a placeholder.

10 Marketing and claims landscape

Narratives that win, clichés that saturate, claims that endanger

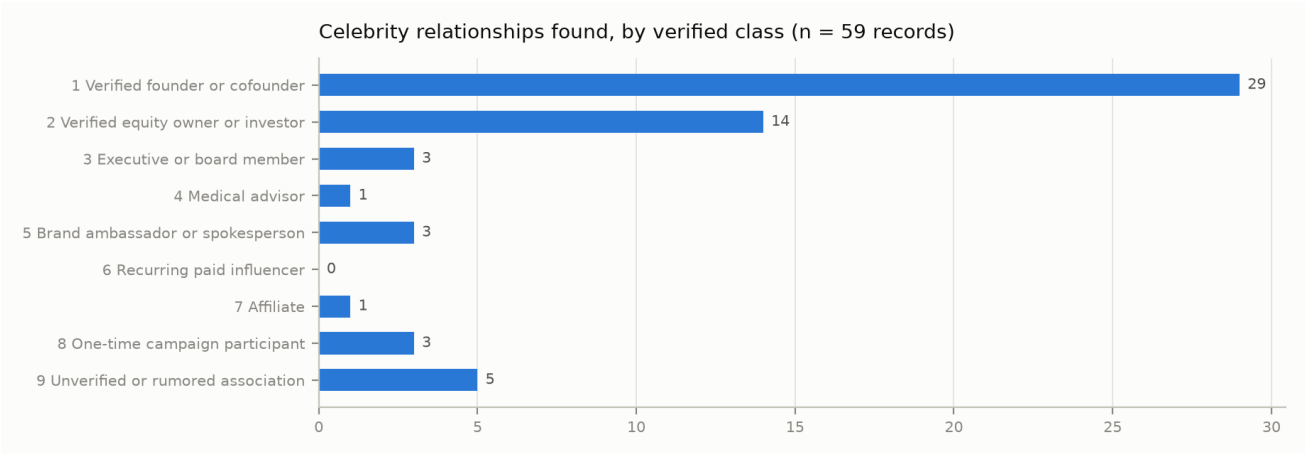
Winning narratives (adopt)	Overused clichés (avoid)	Unsupported or risky claims (never)
Clinician-led, data-driven care; transparent flat pricing; 'we wait for FDA clearance'; labs-first safety; plain-language education (Huberman/Attia-style)	'Optimize', 'biohack', 'unlock', 'peptide stacks', 'anti-aging breakthrough', 'doctor-designed' without names, physique imagery, scarcity timers	'Same as Ozempic'; implying FDA approval of compounded products; healing, longevity or fat-loss outcomes for BPC-157/TB-500; 'natural GLP-1'; results guarantees; typicality-free testimonials; RUO disclaimers as a shield

Acquisition channels observed

Channel	Observed use	Assessment
Organic search and educational content	Hone, Marek, Ivím, Function, telehealth blogs; SSRP for clinicians	Highest durable value; E-E-A-T requires named reviewers
Podcasts and founder channels	Marek (MPMD), Ways2Well (JRE), AG1/Onnit cap tables, Huberman/Momentous	Repeated long-form recommendation converts; scrutiny attaches to host
Paid search	Certified telehealth advertisers (LegitScript required for Rx-related ads)	Gate for Reserve Clinic; apply in Phase 0
Meta / TikTok / YouTube	Hims 2025 Super Bowl (+650% traffic, third-party); TikTok banning retatrutide promotion; Meta health-data restrictions limit optimization	Creative-led broad targeting; non-health optimization events
Affiliates and creator codes	RUO vendors (cited by FDA), consumer supplements (10–30% commissions)	Non-Rx and education only; disclosed
Referral programs	Common in DTC supplements	Membership/non-Rx only; never for prescriptions
Practitioner referrals	Belmar/Wells clinician education; A4M/SSRP	Pursue education partnerships without per-Rx fees
PR and press	Function, Superpower, Neko funding stories; peptide-boom features	Standards-first narrative and medical spokesperson
Email / SMS	Universal	Benchmarks (recalled): email open 35–50%, click 1–2%; SMS CTR 6–10%

Source / label: research/01 and 02 (acquisition_channels_observed, messaging_claims); research/03 (Hims traffic spike, third-party); research/05 (platform policies, third-party reports); research/06 (channel benchmarks, recalled/unverified). Ad libraries were not accessible.

11 Celebrity-founder findings



Source / label: 04_Celebrity_Founder_Database.xlsx; research/03_celebrity_database.json. Classification by evidence, not by advertising appearance; follower counts approximate and to be re-verified.

Verified peptide-direct relationships

Company	Person	Class	Evidence (label)
BioLongevity Labs (RUO manufacturer)	Jay Campbell, Hunter Williams, Josh Felber	1 Founder/cofounder	Forbes Jul 2026 coverage; company statements (third-party / company-reported)
Fountain Life (longevity clinic; peptides, NAD+)	Tony Robbins, Peter Diamandis	1 Cofounder	Company and press; \$18M raise Aug 2025 (third-party)
10X Health / The Ultimate Human	Gary Brecka (former), Grant Cardone	1 Cofounder / 2 Investor	Litigation records 2024-25 (third-party); key-person case
Ro (GLP-1 telehealth)	Serena Williams	5 Ambassador	PR Newswire Aug 21, 2025 (verified); husband Alexis Ohanian is investor and board member
Noom	Rebel Wilson	5 Ambassador ('Chief Wellness Ambassador')	GlobeNewswire Sept 2025 (verified)
Lemme ('GLP-1 Daily' supplement)	Kourtney Kardashian Barker, Simon Huck	1 Cofounder	WWD; class actions CA/NY 2025 (third-party)
Rhode (peptide lip treatment)	Hailey Bieber	1 Founder	\$1B e.l.f. acquisition May-Aug 2025 (verified)
Function Health (diagnostics)	Mark Hyman (cofounder/CMO); Damon, Efron, Hart, Pascal (investors)	1 / 2	Fierce, Modern Healthcare (third-party)
Superpower	Logan Paul, Steve Aoki, Vanessa Hudgens, Giannis Antetokounmpo	2 Investor	Series A announcement Apr 2025 (third-party)
Ways2Well (peptide telehealth)	Joe Rogan (listed as client; founder is repeat JRE guest)	9 Unverified	AP/CNN Nov 2025 (third-party)
Vital Proteins	Jennifer Aniston	3 Executive title (CCO) attached to paid spokesperson role	WWD; NBC (third-party)
Blueprint	Bryan Johnson	1 Founder	NYT 2025 reporting on losses (third-party)

Lessons that shape the Reserve Clinic design

- **Advantage is real when ownership, repetition and substance coincide.** Rhode (peptide lip SKU launch hero, \$1B exit), AG1 and Onnit (podcast-host cap tables), Function Health (investor halo to \$2.5B) and Prenuvo (unpaid Kim Kardashian post cited in a \$120M raise) show measurable effects. Oprah's WeightWatchers arc priced one celebrity at roughly a quarter of market cap on exit (Feb 2024).
- **Involvement is cosmetic when the face is rented.** Ladder reached only ~\$4M revenue with >180M combined followers; SKKN by Kim folded in 2025; investor cohorts announce and disappear.
- **Trust transfers fastest through medical credibility and breaks hardest there.** Prenuvo/Kardashian and 10X/Brecka both converted and both drew physician rebuttals; Goop's 2018 \$145K settlement and Kim Kardashian's 2022 SEC \$1.26M order set the disclosure and substantiation floor.
- **Key-person risk must be contracted for.** TB12's rebrand after the Brady-Guerrero split, 10X's \$100M/\$13M cross-suits, and Bulletproof's managed decoupling are the templates: earnouts, morals and PED-disclosure clauses, testimonial ownership, and equity in operating cofounders with domain credentials.
- **Peptide launches will be read as MAHA-adjacent in 2026.** Press ties the FDA reversal to influencer demand; any celebrity peptide brand inherits that political framing and the scrutiny that follows.

Source / label: research/03_celebrity_database.md sections 3.1–3.7 with sources by record.

12 Reserve Clinic positioning

Four territories were evaluated. Each is viable; one is recommended.

Territory	Target customer / problem	Core promise and reasons to believe	Personality, sample headline	Advantages	Risks
A. Standards-first premium longevity (recommended)	30–55, affluent, peptide-curious, distrustful of hype; problem: no trustworthy front door	'Peptide health, done properly.' Named clinicians; licensed pharmacies; FDA-status badges; evidence grades; all-in pricing; public exclusions	Calm, confident, editorial. 'We'll tell you what we won't sell.'	Defensible against both budget clinics and generalists; aligns with enforcement direction; works in Scenario B	Slower than hype-driven growth; requires clinical bench
B. Modern men's health with peptides	Men 30–50 already in TRT/ED telehealth	'Beyond TRT': integrated hormone and peptide care	Direct, masculine	Large proven persona (Hims, Hone)	Crowded; price-led; less differentiation; excludes women
C. Luxury concierge health	High-net-worth; problem: time and access	White-glove clinician access, labs, priority	Discreet, exclusive	High ARPU; Fountain Life analog	Small; celebrity may skew 'lifestyle over medicine'; low scalability
D. Evidence-first education platform with commerce	Broad curious audience	'Understand peptides before you buy'	Teacherly	Lowest regulatory exposure; SEO moat	Monetization limited; risk of becoming a referral site

Why Territory A

It is the only territory that converts the category's weakness — distrust — into the brand's asset, survives either direction of FDA rulemaking, and does not depend on a celebrity to be credible. Territory B is the fallback if research shows demand is overwhelmingly male and price-sensitive; Territory D is the fallback if counsel restricts the formulary so far that programs are not viable at launch. The brand should never be built on unsupported clinical outcomes or on the assumption that celebrity awareness creates trust; the celebrity research shows it does not.

Brand architecture

- Masterbrand: Reserve Clinic. Sub-labels: Programs (clinician-led), Library (education), Standards (governance), Shop (non-Rx).
- Design system: editorial serif wordmark, humanist sans UI, ink/bone/moss/brass palette, WCAG 2.2 AA; no syringes-as-hero, no before/after, no celebrity imagery without contract.
- Voice: plain language, honest about uncertainty, specific about what is included and excluded.

13 Scott Disick — proposed role

Verification status: identity verified (Scott Michael Disick, b. May 26, 1983; The Kardashians on Hulu; Talentless apparel). No public evidence of any relationship with Reserve Clinic or with any peptide company was found. Everything below is a proposed structure contingent on his interest, his representatives, his Hulu and existing endorsement obligations, and counsel.

What the research found

Finding	Detail	Label
Audience	~27M Instagram; ~5.7M X; small TikTok count reported (verify handle)	Search snippet
GLP-1 association	Confirmed Mounjaro use on The Kardashians (Feb 2025: 'I'm not embarrassed I took it'; ~30 lb lost); a Yahoo headline reports him promoting a GLP-1 medication — brand, date and paid status unverified	Verified / third-party (headline)
FTC history	On the FTC's Sept 6, 2017 list of 21 influencers sent warning letters on undisclosed material connections; 2016 Bootea caption incident; TINA.org database of his posts	Verified
Reputation	Documented addiction and rehab history; frequent tabloid coverage; 2022 car crash	Verified (public reporting)
Commercial	Talentless (2018); IN COMMON Beauty partner (2021); reported 'multiple vitamin companies' (unnamed); est. net worth ~\$45M	Third-party
Rights holders	Hulu cast agreement (two-season renewal 2024; S7 aired Oct 2025); agency not identified; trademarks not queryable	Blocked / inference

Proposed responsibilities and terms (subject to definitive agreements)

Area	Proposal [SR]
Responsibilities	Brand vision and creative direction input; media and launch awareness; community and content (founder-led series, disclosed); partnerships introductions; customer insight sessions. No involvement in clinical decisions, formulary, or medical content.
Deliverables and time	Defined content days per quarter (e.g., 4–6), launch-week availability, quarterly creative review; approvals within agreed SLAs.
Media obligations	Approved scripts only; all health statements medically reviewed; no personal outcome claims that are not typical; no discussion of prescription products beyond approved language.
Approval rights	Consultation on brand creative; no veto over medical, compliance or pricing decisions.
Morality and reputation	Mutual morality clause; suspension and termination triggers; crisis-communications cooperation; PED/medication disclosure obligations relevant to testimonials.
Exclusivity	Category exclusivity in peptide/longevity/telehealth for the term; resolution of any existing GLP-1 endorsement conflict as a condition precedent.
Compensation scenarios (hypothetical)	Cash retainer; equity with vesting tied to deliverables; capped royalty on net revenue; or mixed. Modeled in 07_ReserveClinic_Financial_Model.xlsx (Celebrity Economics). No terms have been offered or accepted.
FTC disclosure	Ownership and compensation disclosed clearly and conspicuously in every endorsement (16 CFR Part 255, 2023); disclosure page on site; training and monitoring.
Name and likeness	Licensed NIL with scope, territory, term, approved uses, and quality control; no use before signature.
Termination and post-termination	Wind-down period for content removal; survival of disclosure obligations; buy-back or forfeiture of unvested equity; non-disparagement.

Source / label: research/04_principal_verification.md sections 1.1–1.8; FTC 2017 press releases (verified); E! Online and others Feb 27, 2025 (search snippets).

14 Dr. Michael Azziz — proposed role

Verification status: no physician with the exact spelling 'Michael Azziz' was located. The most plausible candidate (~60% confidence, analyst inference) is Michael (H.) Aziz, MD, a New York internist (515 Madison Ave; Lenox Hill attending; author of The Perfect 10 Diet and The Ageless Revolution, 2025) whose practice markets peptide therapy, hormones and TRT. Dr. Ricardo Azziz (reproductive endocrinology) and Michael M. Aziz (Allegheny Health Network) are different people. NPI Registry, New York State Education Department, ABIM and PubMed checks were blocked; no disciplinary reports surfaced in the limited searches. The project owner must supply the intended physician's legal name, NPI and license state before any public reference.

Credential requirements before appointment

- Active, unrestricted license in the affiliated practice's home state and in each launch state (or supervision structure per counsel); board certification claim verified (ABMS/ABIM); NPI and DEA (if applicable) verified; malpractice history and disciplinary lookups in all states; peer references; conflict-of-interest disclosure (ownership in pharmacies, RUO vendors, supplement brands, affiliate income).

Proposed responsibilities and protections

Area	Proposal [SR]
Clinical governance	Chairs the clinical governance committee; owns formulary policy keyed to FDA 503A status with change control; approves protocols; reviews adverse events; sets provider standards and credentialing.
Medical education and content	Medical reviewer of record for library and program pages; review dates published; sign-off on all health claims in marketing.
Clinical independence	Employed or contracted by the affiliated professional entity, not the brand/MSO; compensation fixed or time-based; expressly not tied to prescription volume, enrollment or revenue; documented authority to decline or restrict offerings.
Conflicts of interest	Disclosure and recusal policy; no financial interest in dispensing pharmacies or suppliers; no affiliate income from peptide vendors.
State licensure limits	Practices only where licensed; state medical directors added as expansion requires; telehealth modality rules per state.
Professional liability	Individual and entity coverage; tail coverage; incident reporting.
Content approval	Final say on medical accuracy; compliance lead has final say on legal claims; either can block publication.
Succession	Named deputy medical director by Phase 2; documented handover; key-person insurance considered.
Public role	May be presented publicly only after verification and agreement; title matches credentials; biography sourced.

Medical decisions must remain independent of celebrity, sales, or marketing pressure. This is both a regulatory necessity (corporate practice of medicine; fee-splitting; anti-kickback principles) and the brand's core promise.

Source / label: research/04_principal_verification.md section 2; research/05_regulatory_brief.md (CPOM, fee-splitting; analyst inference pending counsel).

15 Recommended business model

Criterion	1. Education and wellness commerce only	2. Telehealth/MSO with affiliated practice and licensed pharmacies	3. Premium membership + telehealth/MSO + compliant commerce (recommended)
Revenue streams	Non-Rx products, affiliate (non-RUO), courses, sponsorship	Program/consult fees (structure per counsel), labs, MSO management fees	Membership + programs + labs + non-Rx + education
Customer experience	Learn and buy; no care	Care pathway; episodic	Continuous relationship: learn → screen → care → monitor
Gross margin	35–65% product	50–65% blended (pharmacy, clinical, shipping)	50–60% blended; membership lifts mix
Operational complexity	Low	High (clinical, pharmacy, state law)	High
Required partners	3PL, suppliers	Telehealth/EHR vendor, professional entity, 503A/503B pharmacies, labs, ID verification, high-risk processor	All of model 2 plus community/content
Regulatory exposure	FTC claims, DSHEA, privacy (CHD laws)	CPOM, fee-splitting, telehealth, pharmacy, FDA status, HIPAA, ROSCA	Model 2 plus auto-renewal on membership
Time to launch	3–4 months	8–10 months	8–10 months (membership can launch earlier)
Capital to month 12 [FA]	\$1–2M	\$3.5–5M	\$4.8M base (\$12.9M conservative)
Geographic scalability	National	State-by-state	State-by-state for Rx; national for education, membership, shop
Technology	Shopify + CMS	Portal + vendor clinical core	Portal + vendor clinical core + Shopify
Acquisition implications	Content and creators; ad platforms permissive	LegitScript certification; health-ad restrictions	Education funnels de-risk paid dependence
Principal risks	Thin differentiation; RUO temptation	Regulatory reversal; enforcement; clinical capacity	Same, plus membership churn

Revenue architecture (legally supportable streams only)

- Membership fees (education, navigation, clinician Q&A, lab coordination, member pricing on non-Rx).
- Program and consultation fees collected in the structure counsel prescribes (the affiliated practice may need to be merchant of record for professional fees; the MSO charges fair-market-value management fees; pharmacies bill medication or the practice does, per state law).
- Compliant non-prescription commerce; educational products; clinician-led group programs; corporate wellness limited to education and testing.
- **Flagged for counsel:** referral fees, pharmacy rebates, prescription-volume incentives, percentage-of-collections management fees in strict CPOM states, and any revenue share with prescribers or celebrities tied to prescriptions.

Source / label: 06_ReserveClinic_Launch_Plan.md; research/05_regulatory_brief.md; financial model. Structure is analyst recommendation pending counsel.

16 Customer experience

The experience is designed around four promises: understand before you commit, a real clinician decides, nothing is hidden, and help is always reachable.

Stage	What the member experiences	What happens behind the scenes	Safeguards
First visit and education	Homepage answers 'what is this and is it for me'; library with FDA status and evidence grades	CMS with medical-review workflow; analytics without health data	Educational disclaimers; reviewer names and dates
Eligibility	3-minute quiz; consented; clear outcome (proceed / not a fit with reasons)	Red-flag rules; state gating; CHD consent logged	No diagnosis; no guarantee of prescription
Intake and consultation	Account, ID check, state consent, history; async review within 48h or video where required	Vendor EHR; licensed clinician in member's state	Independence statement; informed consent; AE instructions
Plan and payment	Clinician plan with all-in price; auto-renewal terms; single confirmation	Stripe Billing; consent versions stored	ROSCA-grade disclosure; 2-click cancel
Fulfillment	Licensed pharmacy ships with tracking; onboarding sequence on use, storage, side effects	e-Rx to 503A/503B; cold chain; status webhooks	Recall procedure; pharmacy disclosure
Monitoring and refills	Day-7 and day-30 check-ins; labs cadence; refill request with short check-in	Clinician review; hold if concerns	Refill criteria; escalation
Support and cancellation	Support hours and SLAs; report-a-side-effect 24/7; pause or cancel in account	Non-clinical vs clinical routing	AE logging; clinician notified on discontinuation

Source / label: 05_ReserveClinic_Website_Strategy.md sections 4-5 [SR].

17 Website concept

ReserveClinic.com should feel premium, confident, modern, medically credible, educational, discreet, human, transparent, easy to navigate and mobile-first. The full specification (principles, design system, 16 page specifications, 12 user flows, 8 wireframes, draft copy, technology comparison) is in 05_ReserveClinic_Website_Strategy.md; the essentials follow.

Design-system direction

Element	Direction
Typography	High-contrast serif wordmark and headings (open-license faces such as Fraunces or Instrument Serif); humanist sans for UI (Inter or Geist)
Palette	Ink #14171C, Bone #F6F3EE, Deep Moss #1F3D33 (single accent), Brass #B08D57 (highlights), Signal Red for warnings only; all pairings WCAG 2.2 AA
Photography	Natural light; real clinicians and consenting members; no syringes-as-hero; no before/after; no celebrity imagery without contract
Components	FDA-status badge; evidence-grade badge; clinician card with license states; all-in pricing table; eligibility stepper; consent modal; adverse-event banner; auto-renewal disclosure block; state-availability selector; cookie/CHD consent manager
Accessibility	WCAG 2.2 AA; keyboard-operable quiz and checkout; captions; automated plus manual screen-reader testing pre-launch and quarterly

Draft homepage copy (requires legal and medical review)

Peptide health, done properly. Independent clinicians decide what's right for you. Licensed US pharmacies fill it. We explain the science in plain language and tell you what we won't sell. [Check eligibility] [Explore the library] — Licensed pharmacies · Independent clinician review · Third-party tested · Transparent, all-in pricing. What we won't do: we do not sell research-use-only peptides, we do not promise results, we do not hide fees or trap you in subscriptions. Prescription products are available only after a consultation with an independent licensed clinician and only where clinically appropriate. Compounded medications are not FDA-approved. Not available in all states.

18 Sitemap and user flows

/	Homepage	/learn, /learn/{slug}	Journal
/how-it-works	Rx path vs non-Rx path	/shop, /shop/{slug}	Non-Rx commerce (standard cart)
/programs	Health-goal hub	/membership	Membership
/programs/recovery metabolic vitality performance sexual-health skin		/faq /support /contact	
/peptides, /peptides/{slug}	Library (FDA status + evidence grade; 'offered by Reserve Clinic?')	/state-availability	Rx programs by state
/science	Science & Standards	/account	Member dashboard
/medical-team	Named clinicians, licensure	/account/program	subscriptions labs orders privacy
/pricing	All-in pricing, cancellation	/legal/privacy consumer-health-data terms telehealth-consent shipping-returns accessibility notice-of-privacy-practices disclosures (material connections)	
/eligibility	Consented quiz → intake or educ.		
/labs /safety /evidence			
Rule: Rx programs never expose 'Add to Cart'. Legal pages are published by the correct entity (brand/MSO vs practice vs pharmacy).			

Primary flows

Flow	Path	Key controls
Visitor → education	Home → library teaser → ingredient page → related program → eligibility	Reviewer credentials; educational disclaimer
Visitor → eligibility	Program → quiz intro → CHD consent → goal, state, age, red flags, meds → contact → outcome	Decline path; disqualification with education; state waitlist
Eligibility → intake	Account (passkey/MFA) → ID verification → state telehealth consent + NPP → history → labs if required → consult-fee authorization	Versioned consents; PHI in vendor EHR only
Approval → payment → fulfillment	Clinician decision → plan and all-in price → auto-renewal consent → payment → e-Rx → pharmacy → cold-chain ship → onboarding → day-7/30 check-ins	ROSCA-grade disclosure; pharmacy verification; tracking
Non-Rx purchase	Shop → PDP → cart → Shopify checkout → 3PL	DSHEA/cosmetic claims only
Membership	Plan → account → auto-renewal consent → payment → dashboard	Renewal reminders; cancel in ≤2 clicks
Labs	Clinician order → site or kit → results to EHR → clinician review → dashboard notification	Results never emailed; abnormal-result outreach
Refill	Dashboard → short check-in → clinician approve/adjust/hold → pharmacy	Refill criteria documented
Cancellation	Subscriptions → pause/change/cancel → confirm → email	One optional pause offer; clinician notified for Rx
Adverse event / urgent	Any page banner → triage → severe: 911 guidance + on-call; non-severe: clinician within 24h → logged; MedWatch where appropriate	Pharmacovigilance SOP; pharmacy notified
Support	Chat/email/phone in hours → clinical questions routed to practice (HIPAA channel)	First response <4h; AE-related <1h

19 Sample wireframes

Low-fidelity wireframes for the eight key screens are in 05_ReserveClinic_Website_Strategy.md section 6. Two are reproduced here.

Homepage — desktop (left) and mobile (right)

Desktop 1280

Reserve Clinic Programs Library Science Team Pricing [Check eligibility] Account					
Serving CA, TX, FL, NY ... · non-Rx ships nationwide					
Peptide health, done properly. Clinician-led programs, tested products, plain-language science. Rx products require a clinician consultation; not everyone qualifies.			[ambient video, muted, captioned]		
Check eligibility		Explore the library			
✓ Licensed US pharmacies		✓ Independent clinicians		✓ Third-party tested	
✓ All-in pricing					
Choose your goal					
Recovery		Metabolic		Vitality	
How it works: 1 Learn → 2 Check eligibility → 3 Clinician plan					
Library: BPC-157 [Restricted — not offered] · Sermorelin [Compounded] Tesamorelin [FDA-approved] · NAD+ [Compounded] · → all entries					
Medical team: Dr. X, MD (CA, TX) · Dr. Y, DO (FL) Independence statement			What we won't do: RUO · unproven claims forced subscriptions · hidden fees		
Learn: three latest articles (medically reviewed)					
Footer: Legal · Consumer Health Data · Accessibility · Disclosures · Rx disclaimer · 911 notice					

Mobile 390

Reserve Clinic Acct	
Serving 12 states	
Peptide health, done properly.	
[video]	
Check eligibility	
Explore the library	
✓ Pharmacies	✓ Clinicians
✓ Tested	✓ Pricing
Choose your goal	
Recovery	Metabolic ▶
1 Learn → 2 Check → 3 Plan	
Library chips · Medical team	
Sticky: Check eligibility	

Program page, eligibility flow, checkout and dashboard

Program page

Programs > Recovery	
Recovery & tissue support Who it's for / who it's not for	Evidence: B
Check eligibility	Read the science
What's included: consult · clinician plan Rx if appropriate · monitoring · support · shipping	
Candidate ingredients	
Tesamorelin [FDA-approved] [Evidence B]	
Sermorelin [Compounded] [Evidence C]	
BPC-157 [FDA-restricted — not offered]	
All-in pricing table (\$/mo · included · excluded)	
Safety & side effects · Labs required Clinician quote (named)	
FAQ	
Medical emergency? Call 911	
Check eligibility	

Eligibility quiz (one question per screen)

Step 1 of 8 ■■■□□□□□	
Privacy: health questions route you safely. [Consumer-health-data notice] ☐ I consent	
What's your main goal?	
☐ Recovery	☐ Metabolic health
☐ Vitality	☐ Performance
Continue	
Later steps: State → Age → Red flags → Conditions → Medications → Contact (SMS opt-in separate)	
Outcome A: proceed to clinician review [Continue to intake]	
Outcome B: not a fit right now Reasons · education · non-Rx options Not a diagnosis; a clinician decides	

Checkout (Rx) and member dashboard

1 Plan > 2 Pricing & consent > 3 Payment	
Order: Recovery program \$X/mo all-in Next charge date · Cancel in account (2 clicks)	
☐ I agree to the auto-renewal terms above	
Confirm & pay	
Secure · PCI · no insurance billed	
Dashboard	
Program: Active Next check-in: day 30	Next refill: Oct 3 [Request now]
Messages (care team) 1 new	Labs Results ready · [View]
Subscriptions Manage · Pause · Cancel	Report a side effect 24/7
Learn Recommended for you	Privacy & data Download · Delete

20 Technology and operations

Criterion	A. Shopify + external clinical workflow	B. Headless commerce + integrated telehealth	C. Custom member portal over proven clinical infrastructure (recommended)
Time to market	8-12 weeks	16-24 weeks	12-18 weeks
Build cost [FA]	\$60-120k	\$250-500k	\$150-300k
Flexibility	Low (vendor UX owns Rx journey)	High	High where it matters
HIPAA / CHD	PHI with vendor; Shopify not for health inputs	Own HIPAA-eligible infra	PHI in vendor EHR + HIPAA-eligible warehouse; minimal consented CHD on brand site
Pharmacy / labs / identity	Via vendor	Direct APIs	Vendor e-Rx network + pharmacy status API; lab vendor; ID vendor
Subscriptions and payments	Shopify (non-Rx)	Custom	Shopify (non-Rx) + Stripe Billing (programs, membership); high-risk backup processor
Analytics and consent	Vendor	Own	Server-side events with health-data filtering; versioned consent log
Maintenance	Low	High	Medium
Risk	Brand break at handoff; weak differentiation	Highest cost; full regulatory surface owned	Vendor dependence for clinical core (acceptable, auditable)

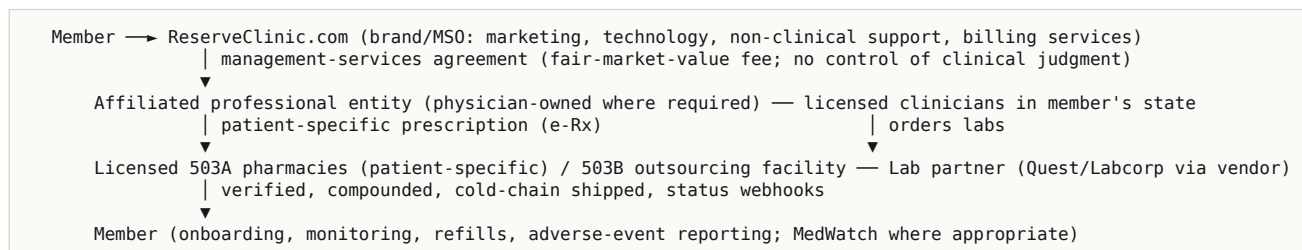
Recommended stack and build/license/integrate split

- **Build:** Next.js marketing site, library, program pages, quiz UI, member dashboard; consent-logging service; analytics pipeline with consumer-health-data filtering.
- **License:** CMS with medical-review workflow (Sanity/Contentful); Shopify for non-Rx; Stripe Billing; identity verification (Persona/Veriff/Stripe Identity); telehealth/EHR/e-Rx and provider network (candidates to diligence: OpenLoop, Wheel, SteadyMD, Healthie with DoseSpot/Surescripts); lab integration vendor; consent-management platform; Klaviyo, Twilio (TCPA consent), HIPAA-eligible messaging; Zendesk.
- **Integrate:** 503A/503B pharmacies with status webhooks and cold-chain carriers; Quest/Labcorp via lab vendor; LegitScript certification; SOC 2 vendors with BAAs.
- **Operations:** pharmacy quality agreements and recall SOP; adverse-event and pharmacovigilance SOP; cold-chain exceptions tracking; certificates of analysis for non-Rx products; cybersecurity program (pen test, incident response, cyber insurance).

Source / label: 05_ReserveClinic_Website_Strategy.md section 8 [SR]; vendor pricing benchmarks in research/06 (recalled/unverified).

21 Clinical and pharmacy partner model

Reserve Clinic (brand and management-services organization) does not practice medicine or dispense drugs. It contracts with an affiliated professional entity that employs or contracts licensed clinicians, and with licensed pharmacies that dispense on valid patient-specific prescriptions. Counsel must confirm the structure state by state; the diagram below is the working hypothesis.



Partner due-diligence framework

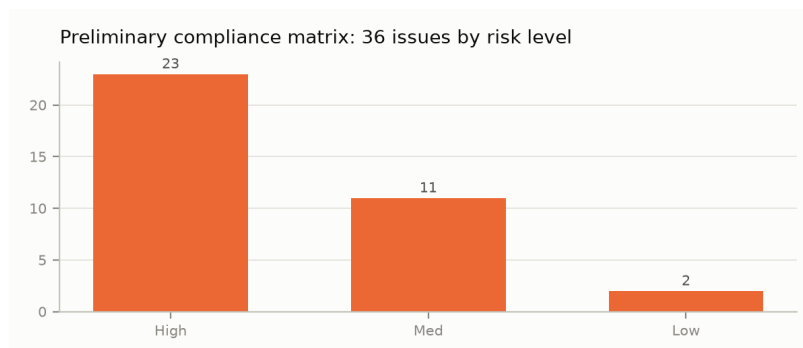
Partner	Diligence items	Contract controls
Medical entity and clinicians	Ownership structure vs CPOM; licenses in each state; board status; malpractice; disciplinary history; telehealth-modality competence	Independence; credentialing; no volume incentives; documentation standards
Telehealth / EHR vendor	HIPAA program, BAA, SOC 2; e-Rx network; state coverage; async/video support; ID verification; uptime	SLAs; data ownership; exit and portability
503A pharmacies	State licensure incl. non-resident; FDA 483/warning-letter history (see Empower, ReviveRx, Boothwyn, ProRx cases); USP 795/797/800; PCAB; testing (sterility, potency, endotoxin); bulks-list compliance	Quality agreement; COA access; recall procedure; adverse-event reporting; audit rights
503B outsourcing facility	FDA registration; cGMP inspection history; product list; capacity	As above plus supply continuity
Testing laboratories and lab partners	CLIA/CAP; draw network; turnaround; result integration	Result handling; abnormal-result escalation
Fulfillment / cold chain	Validated packaging; temperature monitoring; carrier performance	Exception handling; replacement policy
Customer support	HIPAA training; escalation paths	SLAs; QA sampling
Insurance	Professional (entity and clinicians), product, cyber, D&O, general	Certificates; additional-insured status

No specific supplier is recommended in this document; the pharmacies with clean enforcement records identified in the audit (Belmar, Wells, Olympia, Red Rock) are starting points for diligence, not selections.

Source / label: research/02_pharmacy_ruo_consumer.md (enforcement records: FDA pages where surfaced, otherwise third-party); research/05 [analyst inference pending counsel].

22 Compliance framework

The preliminary matrix covers 36 issues (23 High, 11 Medium, 2 Low). The full matrix with owners, professional review, pre-launch actions, ongoing controls and sources is in 08_ReserveClinic_Evidence_Ledger.xlsx (Compliance Matrix) and research/05_regulatory_brief.md. It is research, not legal advice; every item requires review by qualified counsel.



Source / label: research/05_compliance_matrix.json; preliminary risk = enforcement likelihood × severity for a well-run brand (analyst inference).

High-risk issues and pre-launch actions

Issue	Why it matters	Pre-launch action
FDA product status of peptides (503A Category 1/2; 2026 reversal; PCAC July 2026)	Whether a peptide may lawfully be compounded on prescription changed twice since 2023: Sept 2023 Category 2 placements, Feb-Apr 2026 removal of ~14 of 19 peptides from Category 2, and a July 23-24, 2026 PCAC vote recommending BPC-157, KPV, TB-500, MOTS-c, sema...	Adopt a written formulary policy keyed to FDA 503A category status with change control; exclude Category 2 substances and compounded GLP-1s; document clinical-need rationale per patient for any 503A product.
Drug claims / intended use (unapproved new drug and misbranding)	Any statement that a peptide or supplement heals, treats, prevents or cures creates a drug intended use under the FDCA; for unapproved peptides there is no human RCT evidence to substantiate efficacy claims, so claims are unlawful under both FDCA and FTC Act.	Build a claims library with per-claim substantiation files; approved language for 'not FDA-approved' and 'emerging research' framing; ban list of disease/efficacy words.
'Research use only' (RUO) positioning	FDA issued seven warning letters dated Mar 31, 2026 (published Apr 7, 2026) holding that RUO / 'not for human consumption' disclaimers do not negate intended use when marketing describes human effects; Lilly is suing RUO retatrutide sellers; card networks and ...	Prohibit RUO SKUs, drop-shipping and affiliate links to RUO vendors; screen partners and influencers for RUO ties.
Compounded GLP-1s after shortage resolution	Semaglutide (Feb 2025) and tirzepatide (Dec 2024) shortages resolved; FDA issued 55+ warning letters (Sept 16, 2025), ~30 to telehealth companies (Mar 2026) and ~25 more (Jun 2026); Apr 30, 2026 proposal to exclude semaglutide/tirzepatide/liraglutide from the ...	Exclude compounded GLP-1s from formulary; route GLP-1 demand to branded DTC channels (LillyDirect \$299-449, NovoCare \$199-349, TrumpRx ~\$350) via prescribing or affiliate education.
FTC endorsements and influencer disclosures (16 CFR Part 255, 2023)	Material connections (payment, free product, employment, ownership) must be clearly and conspicuously disclosed in the same medium; the advertiser is liable for endorsers' unsubstantiated claims and must monitor.	Influencer contract template with disclosure and claims clauses; disclosure SOP (#ad in-frame, verbal disclosure in video); pre-approval of scripts.
Testimonials and consumer reviews (16 CFR Part 465)	FTC's Consumer Reviews and Testimonials Rule (effective Oct 21, 2024) makes fake, AI-generated, insider or suppressed reviews civil-penalty violations; NextMed (2025) was charged with fake reviews and testimonials from non-customers.	Review-collection SOP (no incentives conditioned on positive reviews, no suppression); verify testimonial authorship; disclose generally expected results.
Before/after imagery	Before/after photos are efficacy claims requiring substantiation and typical-results disclosure; Meta and TikTok restrict them in weight-loss ads; NextMed used before/after pictures of non-customers.	Default ban on before/after in paid media; if used, only real patients with signed consent, dated images, and typical-results disclosure.
Auto-renewal / negative option (ROSCA, FTC rulemaking, CA ARL)	FTC Click-to-Cancel rule vacated Jul 8, 2025 (8th Cir.); FTC ANPRM Mar 11, 2026 (comments closed Apr 13, 2026); ROSCA and state laws (California AB 2863 effective Jul 1, 2025; NY GBL 527-a; others) still require clear disclosure, express consent, easy same-med...	Checkout with all-in pricing, affirmative consent checkbox, one-click online cancel, confirmation and renewal reminder emails, documented refund policy.
Pricing transparency and hidden fees	NextMed order (final Dec 3, 2025) prohibits misrepresenting cost of telehealth services; 'from \$79/month' ads that omit medication, labs, membership terms and early-termination fees are deceptive.	All-in price disclosure standard for every offer; itemize what membership includes and excludes.
Corporate practice of medicine (CPOM) and MSO / friendly-PC structure	CA, NY, TX and NJ prohibit lay control of medical practice; MSO may provide non-clinical services for fair-market-value fees only; 2025-2026 state legislation (e.g., Oregon SB 951) is tightening MSO control features; non-compliance voids contracts and exposes ...	Form PC(s) owned by licensed physician(s); MSA with clinical-independence provisions; succession/continuity agreements compliant with each state; state-by-state CPOM opinion.
Fee-splitting and anti-kickback	Percentage-of-revenue MSO fees and marketing fees tied to prescriptions violate state fee-splitting statutes (CA B&P 650; NY Educ. Law 6531; TX Occ. Code ch. 102; NJ 13:35-6.16); federal AKS applies if federal payors are involved; EKRA and state all-payor stat...	Fixed or FMV-based MSO fees documented by valuation; no compensation tied to prescription volume; affiliate payouts limited to non-Rx products.

Issue	Why it matters	Pre-launch action
State medical practice standards and telehealth modality (sync vs async)	Prescribers must be licensed in the patient's state and meet that state's standard of care; several states restrict questionnaire-only/asynchronous prescribing or require a synchronous encounter before a first prescription; boards have disciplined async GLP-1 ...	State launch matrix (licensure, modality, consent, prescribing rules); default to synchronous video for initial visits; clinical protocols per state.
Provider licensure and credentialing	Each prescriber must hold an active license in the patient's state (IMLC can speed multistate licensure); credentialing, DEA registration (if any controlled substances), NPI, malpractice verification and OIG exclusion checks are expected by boards, insurers an...	Credentialing file per clinician; license map matching launch states; geofenced scheduling.
Pharmacy licensure (non-resident pharmacy; 503A/503B partner diligence)	Partner pharmacies must hold non-resident licenses in every ship-to state, comply with USP <797>/<795> and 503A interstate-distribution limits (5% / MOU), and state-specific restrictions (e.g., Louisiana Board of Pharmacy on semaglutide-class compounding); sta...	Partner due-diligence checklist (licenses per state, inspection history, 483s/warning letters, CoA/potency testing, recall plan); contractual licensure warranties and audit rights.
Rx fulfillment, labeling and patient-specific compounding	503A compounding requires a valid patient-specific prescription and documented clinical need; 'essentially a copy' of an approved drug (e.g., PT-141/bremelanotide, tesamorelin) is prohibited absent a significant difference; FDA's 2025-2026 warning letters cite...	Prescription templates with clinical-need documentation; patient labeling and disclosure that products are compounded and not FDA-approved; no bulk/office-stock dispensing.
HIPAA (clinical data in the telehealth entity and MSO)	The PC is a covered entity and the MSO/platform a business associate; Privacy and Security Rules, BAAs, and the pending 2025 Security Rule overhaul apply; marketing use of PHI requires authorization; pixel sharing of PHI is an OCR and class-action exposure.	BAAs with all vendors; risk analysis; access controls; no marketing pixels on authenticated patient pages; NPP and authorizations.
FTC Health Breach Notification Rule (non-HIPAA health data)	2024 amendments (effective Jul 29, 2024) cover health apps and sites collecting health information; unauthorized disclosure (e.g., ad pixels) is a 'breach' requiring notice; GoodRx (\$1.5M), BetterHelp (\$7.8M) and Cerebral (~\$7M) are precedents; FTC/state suit ...	Data map of quiz/intake/membership health data; consent-gated pixels; no sharing of health data with ad platforms; breach-notification playbook.
State consumer health data laws (WA MHMDA, NV, CT, NY NYHIPA pending)	Washington MHMDA (private right of action) and Nevada/Connecticut laws require a separate consumer-health-data policy, opt-in consent for collection/sharing and signed authorization for sale; New York's NYHIPA was vetoed Dec 19, 2025 but reintroduced (S9269/A1...	Consumer-health-data privacy policy; opt-in consent flows on quizzes and health preferences; no geofencing; data-sale authorization template.
Payment processing (high-risk merchant category)	Stripe prohibits pharmaceuticals/pseudo-pharmaceuticals, nutraceuticals with harmful claims and unapproved research chemicals, and treats telemedicine as restricted; Mastercard's 2026 BRAM update tightened peptide/nutraceutical enforcement; account closures co...	Select a health-vertical acquirer with disclosed MCC and reserve terms; obtain LegitScript before applying; separate MID's for supplements, membership and Rx services.
Ad-platform restrictions (Meta, Google, TikTok, Amazon, LegitScript)	Meta's health-and-wellness data-source restrictions (from early 2025, expanding 2026) block purchase-event optimization; Google requires LegitScript/NABP certification for Rx/telehealth ads and permits recurring-billing Rx promotion for certified merchants fro...	Apply for LegitScript (telehealth/healthcare merchant) and Google/Meta certifications; configure Meta events for restricted account; build measurement on top-of-funnel and modelled conversions.
Professional and product liability; D&O; cyber insurance	Telehealth prescribers need malpractice coverage that includes telemedicine, all licensed states and off-label/compounded products (often excluded); brands marketing compounded products are named in product suits (GLP-1 MDL has 3,200+ federal cases); regulator...	Bind malpractice (\$1M/\$3M per clinician plus entity), products/completed-ops via pharmacy partners with additional-insured status, D&O, tech E&O/media, cyber (\$2-5M); confirm no compounded-drug exclusions.
Data security program	HIPAA Security Rule (and its pending overhaul), state breach laws, CPRA cybersecurity-audit regulations and FTC Safeguards-style expectations require documented controls; breaches trigger HBNR/HIPAA/state notices and class actions.	Risk assessment; encryption, MFA, logging; vendor security reviews; incident-response plan with notification matrix.

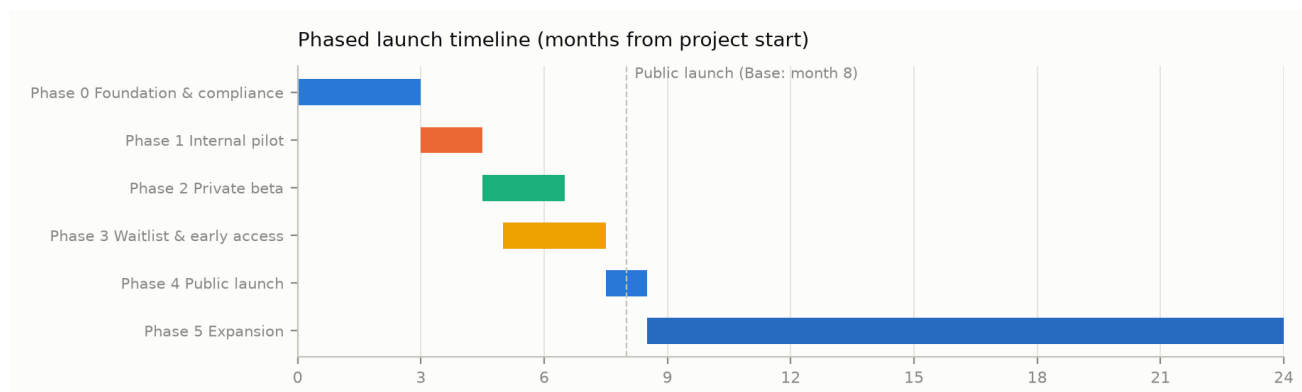
Issue	Why it matters	Pre-launch action
Brand-owner litigation exposure (Novo/Lilly vs telehealth and marketers)	Novo has filed 130+ suits in 40 states, sued Hims & Hers (Feb 2026, dismissed after partnership), and both Novo and Lilly send cease-and-desist letters to med spas/telehealth; Lilly sues RUO retatrutide sellers under Lanham Act theories; claims that compounded...	Brand-name usage guidelines (nominative fair use only); no comparative equivalence claims; cease-and-desist response protocol.

Medium and low issues (summary)

Celebrity / owner endorsements; Telehealth informed consent; Controlled substances (Ryan Haight Act / DEA telemedicine rules); Cold chain, traceability and product quality; Adverse-event handling and pharmacovigilance; CCPA/CPRA and other comprehensive state privacy laws; TCPA / SMS marketing; CAN-SPAM email marketing; Accessibility (ADA Title III / WCAG); Terms of service, privacy policy and refund policy; Trademark, right of publicity and NIL; Age restrictions and vulnerable populations; State AG and UDAP enforcement on wellness/GLP-1 marketing.

Source / label: research/05_regulatory_brief.md sources table (79 rows): FDA, FTC, court and statute references where surfaced (Verified fact); law-firm and trade-press alerts (Third-party report); background items flagged Analyst inference for counsel verification.

23 Launch strategy



Source / label: 06_ReserveClinic_Launch_Plan.md; timing per Base scenario [FA].

Phase	Objectives	Exit criteria	Budget [FA]	KPIs
0 Foundation & compliance (M0-3)	Structure, principals and rights, counsel memos, partners, standards, brand and prototype, analytics, pilot states, advisors	Counsel sign-off; signed clinical and pharmacy agreements; insurance; pilot states	\$350-700k	Checklist items 1-10; prototype SUS ≥80
1 Internal pilot (M3-4)	Test positioning, flows, pricing, support, clinical handoffs	Pricing v1; P0/P1 issues closed; compliance spot-check	\$80-150k	Task success ≥90%; intake→decision ≤48h
2 Private beta (M4-6)	150-300 invited members in 2-3 states; safety, retention, fulfillment	≥200 served; unit economics within 20% of model; AE process validated	\$150-300k	Month-2 retention ≥75%; fulfillment ≤5 days; refunds <5%
3 Waitlist & early access (M5-7)	Owned audience; authority content; podcasts; referral mechanics	Waitlist ≥15k; library live; nurture performing	\$200-400k	Email open ≥45%; organic library sessions ≥25k/mo
4 Public launch (M7-8)	Coordinated multi-channel launch; celebrity audience only if contracted; crisis coverage	Stable operations 4 weeks; CAC and retention within bands; no open P0	\$400-900k (month)	≥1,000 enrollments in 30 days (base); CAC ≤\$350; AE response <1h

Phase	Objectives	Exit criteria	Budget [FA]	KPIs
5 Expansion (M9–24)	States in tranches; programs by evidence; retention and referral; partnerships with guardrails	Sustainable contribution margin; repeatable state playbook	\$150–400k/mo	20+ states by M18; LTV:CAC ≥ 3 ; NPS ≥ 50 ; 0 compliance incidents

Roadmap

30 days	60 days	90 days	6 months	12 months
Counsel engaged; structure memo; principal verification; trademark clearance; vendor RFPs; brand direction; Scenario A/B framework	Term sheets or documented no-go; formulary v1; telehealth vendor selected; pharmacies in diligence; prototype + 20 library entries; analytics/consent live	Entities formed; agreements signed; LegitScript application; pilot states; internal pilot; pricing research fielded	Private beta complete; waitlist $\geq 15k$; 40+ library entries; podcast placements; go/no-go for launch	Public launch done; 10–15 states; 3–4 programs; membership live; ~1,700 active Rx members (base); first Standards Report

24 Channel plan

Channels are structured as owned (website, email, library, community), rented (social, search, media platforms) and borrowed (podcasts, clinicians, creators, partners, press). Every rented or borrowed impression is routed into an owned asset. Prescription volume is never a growth KPI.

Channel	Phase 0–1	Phase 2–3	Phase 4	Phase 5
Website / library (owned)	Prototype; 20 entries	40+ entries; articles	GA	State pages; continuous
Email / SMS (owned)	Consent flows	Waitlist nurture	Launch waves	Lifecycle, retention
Founder / clinician video (owned)	Scripts; medical review	Series 1	Launch content	Ongoing
Podcasts (borrowed)	Target list	5+ placements	Tour	Recurring
Creators (borrowed/rented)	Contracts; disclosure kit	Disclosed seeding	Wave	Ambassadors (disclosed)
PR (borrowed)	Backgrounder	Standards narrative	Exclusives	Quarterly reports
Paid social (rented)	—	Waitlist ads (non-health events)	Scale with guardrails	Efficiency
Paid search (rented)	LegitScript prep	—	Brand + category (certified)	Scale
Practitioner referrals (borrowed)	Advisory clinicians	Pilot referrals	—	Program (no per-Rx fees)
Partnerships (borrowed)	—	—	—	Gyms, clinics, corporate education/testing

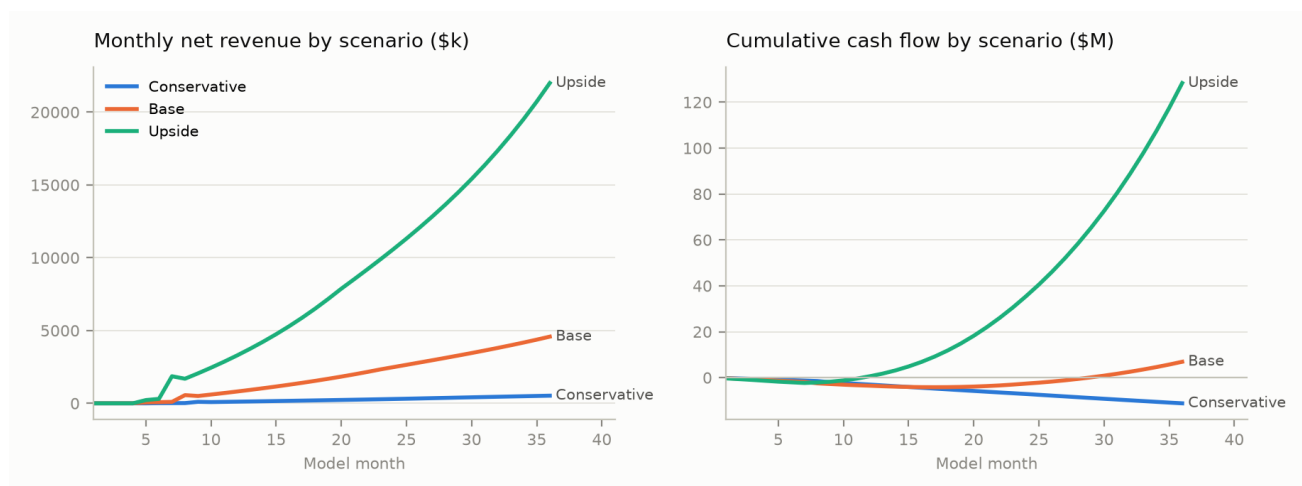
Measurement framework (base-case month-12 targets [FA])

Metric	Target	Metric	Target
Qualified traffic	120k sessions/mo	Gross margin	55-65%
Waitlist conversion (pre-launch)	6-10%	Refund rate	<4%
Quiz start / completion	8% / 65%	Monthly churn (blended Rx)	8-12%
Consultation completion (show rate)	≥80%	LTV : CAC	≥3.0
Medical eligibility rate	50-70% (monitored, not targeted)	CAC payback	≤6 months
Enrollment conversion (eligible → paid)	≥45%	Referral share of new members	≥12%
Blended CAC	≤\$300	NPS	≥50
AOV / program value	\$250-350	Support first response	<4h (AE <1h)
Fulfillment time (median)	≤5 days	Compliance incidents	0

Source / label: 06_ReserveClinic_Launch_Plan.md sections 4-5 [SR/FA].

25 Financial model

The model (07_ReserveClinic_Financial_Model.xlsx) is driver-based and editable: 68 labeled assumptions in three scenario columns feed three 36-month model sheets, a scenario summary, sensitivity grids, hypothetical celebrity-economics structures and an assumptions register with rationale, source, confidence, sensitivity and validation method. Every output is a formula; no result is hard-coded. Nothing here is a forecast of results.



Source / label: 07_ReserveClinic_Financial_Model.xlsx, Model_* sheets. Financial assumptions [FA]; benchmark-informed inputs are labeled recalled/unverified in the assumptions register.

Output	Conservative	Base	Upside
Year 1 / 2 / 3 net revenue	\$444k / \$2.5M / \$5.1M	\$3.4M / \$19.8M / \$42.8M	\$14.7M / \$83.7M / \$194.2M
Year 3 gross margin	34%	50%	60%
Year 1 / 2 / 3 EBITDA	(\$3.0M) / (\$4.1M) / (\$4.0M)	(\$3.4M) / \$1.0M / \$9.7M	\$806k / \$35.2M / \$93.7M
Active Rx members, month 12 / 24 / 36	229 / 706 / 1,303	1,688 / 6,031 / 11,438	6,763 / 24,844 / 52,300

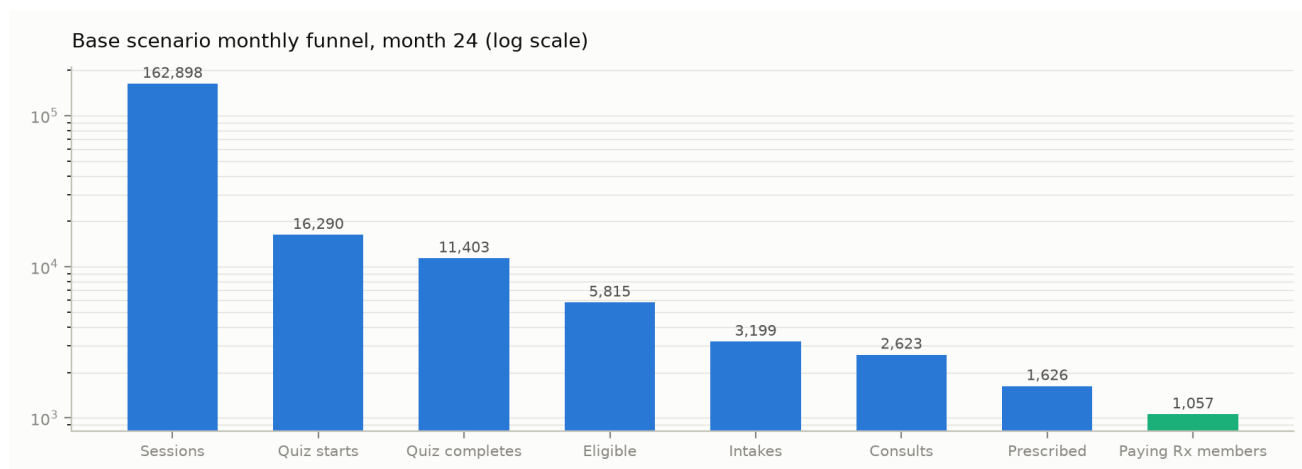
Output	Conservative	Base	Upside
CAC per Rx member, month 24		\$1k	\$313
Contribution per Rx member-month / LTV	\$85 / \$651	\$134 / \$1k	\$174 / \$2k
LTV : CAC, month 24 / payback (months)	0.6x / 13.7	3.9x / 2.3	12.5x / 0.8
Break-even month (EBITDA)	Not within 36 months	18	8
Peak cumulative burn	(\$11.2M)	(\$4.2M)	(\$2.3M)
Capital required (+15% contingency)	\$12.9M	\$4.8M	\$2.7M
Brand-partner cash + royalty, 36 months (Scenario A)	\$1.0M	\$2.9M	\$10.1M

Source / label: 07_ReserveClinic_Financial_Model.xlsx (Scenario Summary), recalculated 2026-09-02. Prescribing rate is a clinical outcome monitored for safety, not a target.

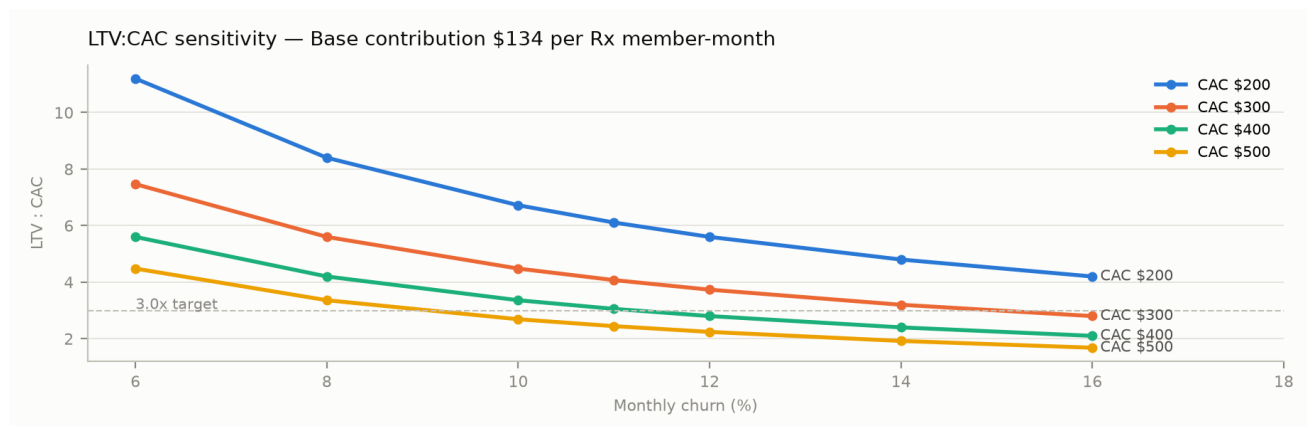
Revenue and cost architecture

- **Revenue drivers:** paid and organic sessions → quiz starts → completions → eligible outcomes (screen and state coverage) → intakes (consult fee) → consults → prescriptions (clinician decision) → paying program members; plus membership sign-ups, lab orders and non-Rx orders with repeat purchases; monthly churn on programs and membership; state coverage rising from 40% to a cap.
- **Costs:** pharmacy/medication per member-month, clinician cost per consult and per member-month monitoring, labs, cold-chain fulfillment, payment fees, refunds, variable support, non-Rx COGS; operating expenses by phase for team, medical leadership, technology, build, legal and compliance, insurance, creative; acquisition (paid, influencer, launch PR) and brand-partner cash and royalty (Scenario A toggle).
- **Working capital:** non-Rx inventory only; prescription products are dispensed by pharmacies.

26 Scenario and sensitivity analysis



Source / label: Model_Base, month 24. Each stage is a labeled assumption; the prescribing stage is a clinical decision.



Source / label: 07_ReserveClinic_Financial_Model.xlsx (Sensitivity). Contribution = program price net of refunds and fees minus pharmacy, monitoring, shipping and support per member-month.

What matters most

Driver	Why it dominates	Validation before launch
Monthly churn on programs	LTV is contribution ÷ churn; moving churn from 11% to 14% cuts LTV by about a fifth	Beta cohort retention; onboarding and check-in cadence
Cost per paid session and funnel rates	CAC per Rx member scales inversely with the product of seven funnel rates	Waitlist and beta analytics; landing-page tests
State coverage	Eligible outcomes scale linearly with covered population	Counsel state map; pharmacy licensure
Program price and pharmacy cost	Together set contribution per member-month	Pricing research; signed pharmacy contracts
Celebrity uplift and cost	Scenario A adds retainer and royalty; uplift on organic traffic is unproven	Brand-lift study in Phase 3; toggle Scenario B in the model

The conservative case is deliberately severe: it assumes weaker funnel rates, higher churn and cost, and that the team and marketing plan continue without cuts. It does not reach break-even within 36 months and requires the most capital; a real operator would re-plan at the Phase 2 exit gate if beta economics track the conservative case.

27 Funding and resource requirements

Bucket (months 0-12) [FA]	Conservative	Base	Upside
Legal, regulatory, insurance	\$250k	\$350k	\$450k
Product, engineering, design	\$350k	\$550k	\$800k
Clinical leadership and governance	\$150k	\$250k	\$350k
Content, brand, production	\$200k	\$350k	\$600k
Paid acquisition and creators	\$400k	\$1.0M	\$2.0M
Team and operations	\$700k	\$1.1M	\$1.6M
Principal compensation (cash portion; Scenario A)	\$0 (Scenario B)	\$300k	\$750k
Working capital and contingency	\$250k	\$400k	\$600k
Year-1 requirement (plan view)	~\$2.3M	~\$4.3M	~\$7.2M
Capital required to peak burn + 15% (model view)	\$12.9M	\$4.8M	\$2.7M

Source / label: 06_ReserveClinic_Launch_Plan.md section 7 (plan view) and 07_ReserveClinic_Financial_Model.xlsx (model view). The two views differ by design: the plan view is a year-1 budget; the model view is cumulative burn to break-even including years 2-3 where applicable.

Team, partners and approvals

Team by launch	Partners	Approvals and certifications
CEO/GM; Medical Director (proposed Dr. Azziz, subject to verification); Compliance & Regulatory Lead; Head of Product & Design; 2-3 engineers; Head of Growth; Content Lead + 2 medical writers; Head of Support; Pharmacy & Ops Lead; fractional CFO; brand partner (proposed Scott Disick, subject to agreement); PR and production agencies	Telehealth/EHR/e-Rx vendor; affiliated professional entity; 2 licensed pharmacies (503A) and 1 503B relationship; lab partner; ID verification; payments (primary + high-risk backup); CMP; 3PL; insurance broker	Counsel opinions (CPOM, fee-splitting, telehealth, pharmacy, privacy, advertising); LegitScript certification; state pharmacy and clinician licensure verification; insurance binding; medical director and compliance sign-off; trademark filing

Hypothetical brand-partner structures (no terms offered)

Structure	Shape	Trade-off
1 Cash retainer only	Fixed annual fee	Front-loads risk before demand is proven
2 Equity only	Fully diluted stake with vesting	Aligned; dilutive; no cash cost
3 Royalty only	% of net revenue	Scales with success; margin drag
4 Mixed (recommended starting point)	Modest retainer + capped royalty + vesting equity tied to deliverables	Balances incentives and cash
5 Performance royalty	Royalty above a revenue hurdle	Pays only for outperformance

Source / label: 07_ReserveClinic_Financial_Model.xlsx (Celebrity Economics); licensing benchmarks recalled/unverified.

28 Milestones

Month	Milestone	Owner	Gate
1	Counsel engaged; principal verification complete; trademark clearance filed; vendor RFPs	GM; counsel	—
2	Term sheets or Scenario B decision; formulary v1; telehealth vendor selected	GM; Medical Director	Founder decision
3	Entities formed; pharmacy agreements; prototype and 20 library entries; analytics/consent stack	GM; Product; Compliance	Phase 0 exit
4	Internal pilot; pricing research fielded; accessibility and security audits	Product; Medical Director	Phase 1 exit
5-6	Private beta in 2-3 states; ≥200 members; AE drill; LegitScript application	GM; Support; Ops	Phase 2 exit
6-7	Waitlist ≥15k; 40+ library entries; podcast placements; creator contracts	Growth; Content; PR	Phase 3 exit
8	Public launch (4-6 states)	All	Go/No-Go complete
12	10-15 states; membership live; first Standards Report; base ~1,700 active Rx members	GM; Ops	Board review
18	20+ states; LTV:CAC ≥3; repeatable state playbook; Series A readiness (if venture path)	GM; Finance	Board review
24	Positive monthly EBITDA sustained (base from month 18); 3-4 programs; corporate education pilots	GM	—

29 Principal risks and mitigations

Risk	Likelihood / impact	Mitigation
FDA reverses or narrows the 2026 peptide reclassification; final rule excludes key substances	Medium / High	Formulary keyed to current status with change control; launch on sermorelin, NAD+, approved drugs; library, membership and non-Rx do not depend on restricted peptides
FTC or state action on subscriptions, testimonials, endorsements or health-data sharing	Medium / High	ROSCA-grade flows; verified reviews only; disclosure program; no health data to ad platforms; compliance lead sign-off on all creative
State CPOM, fee-splitting, telehealth-modality or pharmacy-licensure barriers slow expansion	High / Medium	State readiness checklist; tranche expansion; counsel map; async-friendly states first
Ad platforms or payment processors restrict the account	Medium / High	LegitScript certification; creative-led broad targeting; education funnels; backup high-risk processor; reserves budgeted
Retention below plan (month-1 <85%; churn >12%)	Medium / High	Onboarding cadence; clinician check-ins; labs cadence; pause option; membership layer
Principal controversy or exit (Scenario A)	Medium / High	Morality, exclusivity, termination and post-termination terms; Scenario B parallel; clinicians as durable face
Pharmacy quality event or recall	Low-Medium / High	Diligence on enforcement history; quality agreements; two pharmacies; recall SOP; member communications plan
Adverse event with public attention	Low / High	Pharmacovigilance SOP; 24/7 escalation; medical spokesperson; transparent reporting
Clinical capacity shortfall at launch	Medium / Medium	50% capacity buffer; wave-based waitlist release
Trademark or naming conflict: RESERVE is a widely used mark element, CLINIC is descriptive, and 'clinic' in a business name may be regulated for non-professional entities in some states	Medium / Medium	Clearance search in Classes 5, 35, 44, 25 before spend; counsel opinion on which entity may use the name; fallback naming
Conservative case realized without re-planning	Medium / High	Phase 2 exit gate with unit-economics thresholds; spend discipline tied to payback

Source / label: research/05_regulatory_brief.md; research/03; financial model; analyst inference.

30 Go/no-go requirements

No customer is accepted until every item is complete and signed off in writing by the Medical Director, the Compliance Lead and the GM.

#	Requirement
1	Corporate structure executed; counsel opinion on CPOM and fee-splitting for each pilot state
2	Affiliated professional entity formed; medical director appointed with verified licenses, board status and malpractice coverage
3	Formulary approved: every ingredient classified (approved / compounded-permissible / excluded); restricted bulk substances excluded from compounded offerings
4	Pharmacy partners licensed in each pilot state (verified with boards); 503A/503B roles defined; quality agreements and recall procedures signed
5	Telehealth vendor, EHR, e-Rx and identity verification live; state-specific consents versioned and logged
6	Privacy program: HIPAA policies and BAAs; consumer-health-data policy; consent manager configured; DSAR process tested
7	Claims review of all site copy, ads and creator scripts against FTC Health Products Guidance and Endorsement Guides; disclosures live
8	Auto-renewal compliance: pre-checkout disclosure, affirmative consent, easy cancellation, renewal reminders per state law
9	Payments: processor approval in writing for the business model; backup processor; chargeback plan
10	Insurance bound: professional (practice and clinicians), product, cyber, D&O, general
11	Adverse-event and pharmacovigilance SOP tested; 24/7 clinical escalation staffed
12	Accessibility audit passed (WCAG 2.2 AA); penetration test passed; incident-response plan approved
13	Support staffed to SLA; refund and return policies published
14	Trademark application filed with clearance memo; domain and social handles secured
15	Principals: identity and credentials verified; agreements signed with NIL, morality, exclusivity, FTC disclosure, termination and post-termination terms — or Scenario B activated and all principal references removed
16	LegitScript certification granted, or paid search deferred
17	Written sign-offs: Medical Director, Compliance Lead, GM

31 Recommended next steps

Priority	Action	Owner	Timing	Why it is first
1	Verify the principals: obtain the intended physician's legal name, NPI and license state and run board, ABIM and NPI checks; identify Scott Disick's representatives, the reported GLP-1 promotional post and any category exclusivity; obtain term-sheet interest or activate Scenario B	Founder; counsel	Weeks 1–4	Everything downstream (brand, comp, disclosures) depends on it
2	Engage healthcare regulatory counsel for the structure memo (MSO + affiliated practice), state map, formulary policy keyed to FDA 503A status, and advertising review process	GM; counsel	Weeks 1–6	Determines what can be sold, where, and how revenue flows
3	Trademark clearance and filing for RESERVE CLINIC (Classes 5, 35, 44, 25); confirm domain registrar account; secure social handles	GM; IP counsel	Weeks 1–3	RESERVE-formative marks are common and 'clinic' naming may be regulated; a conflict later is expensive
4	Issue RFPs: telehealth/EHR/e-Rx vendor; two 503A pharmacies and one 503B; lab partner; identity verification; payments (primary and high-risk backup); begin LegitScript preparation	Product; Ops; Finance	Weeks 2–8	Longest lead times; gate Phase 2
5	Fund Phase 0 and start: brand identity, prototype with 20 library entries and eligibility quiz, analytics and consent stack, advisory group, and the pricing research plan	GM; Product; Growth	Weeks 2–12	Builds the owned assets that every later phase compounds on

Decisions requiring founder approval

- Scenario A versus B, and the negotiating envelope for principal compensation (cash, equity, royalty, mixed).
- Positioning territory (A recommended) and the fallback if pricing research or counsel constrains the formulary.
- Pilot states and the state-expansion sequence.
- Capitalization target: base-case capital requirement versus conservative-case cover.
- Whether to pursue the membership layer at launch or after beta.
- Approval of the verification backlog and budget to re-run the blocked research items before any external use of this document.

32 Sources and appendices

The complete evidence register (790 entries with URLs, access dates and labels) is in 08_ReserveClinic_Evidence_Ledger.xlsx. Selected primary and high-value sources are listed below; entries marked (snippet) were seen only as search-result excerpts of the cited page.

Appendix A — Selected sources

Topic	Source	Label
FDA 503A bulks-list update removing peptides from Category 2 (Apr 15, 2026); PCAC Jul 23–24, 2026 recommendations	AJMC; STAT (2026-07-23); Pharmacy Times; Newtropin; DJ Holt Law	Third-party reports
FDA PCAC briefing citing online BPC-157 availability	fda.gov/media/193343/download	Verified fact (surfaced)
FDA RUO warning letters (Mar 31, 2026; Gram Peptides; Wholesale Peptide Jun 17, 2026)	fda.gov warning-letter pages; Health Law Alliance	Verified / third-party
Compounded GLP-1 enforcement (Sept 2025; Mar and Jun 2026); Apr 30, 2026 503B proposal	Venable; Sheppard Mullin; Orrick; Pharmacy Times	Third-party reports
Pharmacy warning letters (Empower 2025; ReviveRx 2025-09-22; Boothwyn 2026-01-16; ProRx 2025-03-04; Tailor Made 2020)	fda.gov warning-letter database entries as surfaced	Verified fact (surfaced)
FTC NextMed final order (Dec 3, 2025); Click-to-Cancel vacatur (Jul 8, 2025); ANPRM (Mar 11, 2026)	ftc.gov; law-firm alerts	Verified / third-party
FTC Sept 6, 2017 influencer warning letters (incl. Scott Disick)	ftc.gov press release; TheFashionLaw; Consumer Reports	Verified
Scott Disick Mounjaro disclosure (Feb 2025)	E! Online; Just Jared; Hola (snippets)	Verified across outlets (snippets)
Michael Aziz, MD practice and books	michaelazizmd.com; US News; Zocdoc; publisher pages (snippets)	Company-reported / third-party
Ro-Serena Williams (Aug 21, 2025); Noom-Rebel Wilson (Sept 2025)	PR Newswire; GlobeNewswire	Verified
Rhode-e.l.f. (\$1B, May–Aug 2025)	WWD; Fortune	Verified
Function Health Series B (\$2.5B, Nov 2025); \$450M financing (Jul 2026)	Modern Healthcare; Radiology Business; press	Third-party
Hims & Hers Q2-2026 revenue; peptide launch plan	Bloomberg (2026-08-10); earnings coverage (snippets)	Company-reported via press
Lemme GLP-1 Daily class actions (2025)	Bloomberg Law; OpenClassActions; SupplySide SJ	Verified (snippets)
Goop settlement (2018); Kardashian SEC order (2022)	Sonoma County DA; sec.gov	Verified
Thorne-P&G (\$3.8B, Aug 2026); Medik8-L'Oréal	Press coverage (snippets)	Third-party
Market-size ranges (peptide therapeutics, compounding, telehealth, longevity, GLP-1, TRT, wellness, supplements, diagnostics)	Grand View Research; Precedence; Fortune BI; McKinsey; KFF (URLs in ledger)	Recalled / unverified

Appendix B — Deliverables

File	Contents
01_Peptide_Market_Landscape.csv	109 companies by segment with legal model, business model, pricing examples, ranks, evidence confidence and source
02_Competitor_Website_Audit.xlsx	Framework, scores (with assessment status), score notes, profiles, journey and medical notes, messaging, channels, rankings, 374 sources
03_Peptide_Product_and_Pricing_Audit.xlsx	136-item product inventory, price bands, monetization patterns, claims classification, regulatory legend, pricing research plan
04_Celebrity_Founder_Database.xlsx	59 classified relationships, summary formulas, lessons, principal watchlist, sources
05_ReserveClinic_Website_Strategy.md	Principles, design system, sitemap, 16 page specs, 12 flows, 8 wireframes, draft copy, technology comparison and stack
06_ReserveClinic_Launch_Plan.md	Six phases with objectives, owners, dependencies, budgets, KPIs, risks, exit criteria; roadmaps; measurement; team; budget; pricing plan; go/no-go; crisis readiness
07_ReserveClinic_Financial_Model.xlsx	Driver-based 36-month model, three scenarios, sensitivity, celebrity economics, assumptions register
08_ReserveClinic_Evidence_Ledger.xlsx	790 evidence entries, principal verification, market size, benchmarks, compliance matrix, assumption traceability, verification backlog

Appendix C — Verification backlog (priority 1)

- Re-run website audits from rendered pages with screenshots for the top 30 companies.
- Verify Hims & Hers, LifeMD and WW 2025–2026 filings; Function, Superpower and Lifeforce funding; syndicated market reports.
- Open FDA 503A bulks-list page and PCAC July 2026 materials; confirm any final rule.
- USPTO clearance for RESERVE CLINIC; WHOIS for reserveclinic.com; social handles.
- NPI, state board and ABIM checks for the intended physician; identify the reported Disick GLP-1 promotional post and its terms.